



HIGHER HEALTH

Higher Education & Training: Health, Wellness and Development Centre

ANNUAL REPORT
2020/21

— +



HELPLINE

BE SAFE



mobile clinics



use
contraception



TALK TO A
COUNSELLOR



higher education
& training
Department:
Higher Education and Training
REPUBLIC OF SOUTH AFRICA



UNIVERSITIES
SOUTH AFRICA



College Principals





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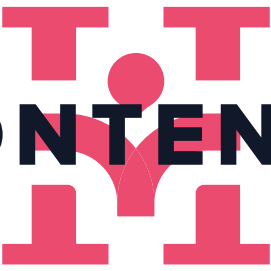
December 2021

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BE SAFE





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MESSAGE FROM THE CHAIRPERSON

Higher Health has emerged from a tumultuous year with an impressive record of leadership and service, and an enhanced reputation across the higher education and training sector. The organisation stepped forward as the COVID-19 pandemic assaulted South Africa and put its experience and resources at the disposal of the sector.

Anyone who has dealt with a major disease outbreak will tell you that moderation is not an option: you need to act quickly and give the effort everything you can. That is the principle that Higher Health followed in relation to COVID-19, collaborating with a wide range of organisations and repeatedly taking on new challenges.

Over the months the focus moved from formulating guidelines and protocols to training staff and student volunteers to implement the protocols, and onward to solving practical problems of implementation and containing cluster outbreaks of COVID-19 at various institutions. As the financial year drew to its close, Higher Health was immersed in the complexities of planning the sector's vaccination programme which would become operational later in 2021.

Massive though the COVID-19 effort was, somehow Higher Health retained a measure of balance. The campus health and wellness programmes that existed before COVID-19 were substantial: they addressed serious health issues and had to be sustained.

The trust of partners and funders who had invested long-term in Higher Health had to be preserved. In short, attention had to be paid to the organisation's sustainability beyond the COVID-19 crisis.

Higher Health achieved a remarkable degree of continuity in its existing health and wellness programmes, despite the complete or partial absence of students from campuses

PROF WIM DE VILLIERS
CHAIRPERSON





at various times during the academic year. To some extent, it seems that universities and technical and vocational education and training (TVET) colleges have become hard-wired to deliver Higher Health interventions – these are no longer up-and-coming interventions but ongoing services. However, allowance must also be made for the ability of Higher Health, universities and colleges to adapt their style of programme operation to survive in the strange world of 2020/21.

The weight of the year's added responsibilities was born by Higher Health CEO Prof Ramneek Ahluwalia and the entire Higher Health team, from senior management down to campus health and wellness coordinators (CHCWs) at institution level. The impact on the Board was relatively light but adjustments were required to keep abreast of the dramatic growth of the organisation in terms of scope of work and financial resources at its disposal. We became more agile, meeting more frequently, sometimes on an ad hoc basis between scheduled meetings.

Higher Health has really been built on the goodwill of countless partners. In addition to its primary partners – public universities and TVET colleges across the country – it owes much to an array of organisations, including domestic and international funding organisations, national and provincial health departments, and non-governmental organisations. These entities lent their support simply because they shared our vision of improving the lives of the country's young people. Higher

Health has valued the opportunity to reciprocate this generosity by lightening the COVID-19 load of many of our partners in the higher education and training and health sectors.

We are grateful to the Minister of Higher Education, Science and Technology, Dr Blade Nzimande, and his management team for the confidence they have shown in Higher Health. This ensured Higher Health a seat at the table when important policy decisions on the national COVID-19 response were being shaped and allowed the sector to act with greater insight.

The lessons of this demanding period have yet to be fully digested. But it is already clear that intense and strategic focus, a determination to build solutions from the resources at hand, and the courage to do some things differently have played a part. As we reflect more closely as an organisation, I believe the maturity we have gained can be deepened and carried into the future.

It seems almost superfluous to thank people for something they do from a deep sense of purpose and commitment. But recognition remains important and I want to say to Prof Ahluwalia and every member of staff that your extraordinary effort, and the sacrifices this entailed, are understood and deeply appreciated. To my fellow Board members, I extend my thanks for their endeavours in guiding the organisation through an extraordinary time.

MESSAGE FROM THE DEPUTY CHAIRPERSON

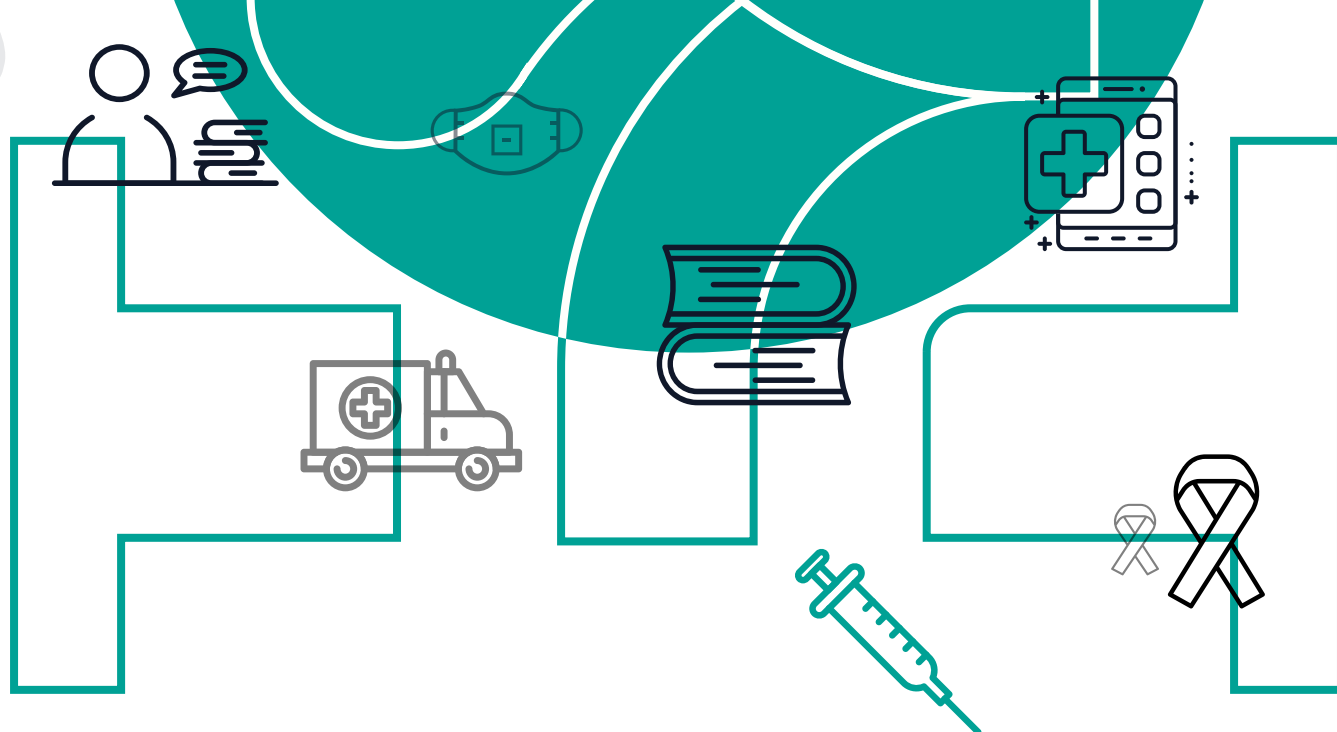
For educators in the post-school education and training sector, the burning question of 2020 was whether the academic year would be sacrificed to the COVID-19 pandemic – or could it be saved? In many instances, particularly among technical and vocational education and training (TVET) colleges, this depended critically on resuming on-campus teaching and learning. But how could an acceptably safe return to campus be achieved?

The answers emerged, in large part, from Higher Health. Even those who knew the organisation well were amazed at the speed with which Higher Health moved and the capacity it was able to mobilise to assist campuses to re-open. For years Higher Health had worked in the background, patiently building health programmes to support students in their pursuit of an academic or vocational qualification. And suddenly the organisation was propelled to the forefront of the sector's response to an unparalleled crisis.

As head of a TVET college and a member of the Board of Directors, I experience Higher Health in a variety of ways: as a partner, as a beneficiary and as a custodian. From whichever perspective I consider things, I come to the conclusion that Higher Health's role in the last year has been irreplaceable.

We need to appreciate, however, that the organisation's contribution did not materialise from nowhere. The foundation had been laid over the past few years as Higher Health expanded its vision and its support structure for the delivery of health





programmes by TVET colleges and universities. Rebranding itself as a broad, aspirational health and wellness programme, instead of an HIV-focused initiative, did a lot to improve its appeal to students. Increasing its hands-on capacity to deliver activities at TVET colleges was also a crucial move.

This report describes what Higher Health did in 2020 and the scale on which it operated. I would like to add a few words on how this was done. The outstanding qualities of the organisation were its ability to provide answers that were based on the best available COVID-19 science and its responsiveness at all times. This held true, whether

one was seeking guidance on some complex matter from the Chief Executive or practical assistance from a campus health and wellness coordinator.

In the past year we have all learned important lessons about the sheer value of health in all our endeavours. We have come to realise first-hand how our own behaviour can either jeopardise or safeguard our lives and the lives of others. We have seen how people who had pre-existing health conditions were much more seriously affected by COVID-19.

We now appreciate much more the messages that Higher Health has been preaching – about prevention being better than cure and health being a holistic matter. Maybe some ears were closed to these arguments before COVID-19 made its appearance. But I truly hope – and believe – that Higher Health's meaning is now clear to many more people and a supportive climate exists for its work going forward.

MR SANELE MLOTSHWA
DEPUTY CHAIRPERSON



CHIEF EXECUTIVE OFFICER'S OVERVIEW

OUR VISION

To be a primary instrument in our public post-school education and training system for improving students' success rates and study completion by enhancing their overall health and psychosocial wellbeing.

PROF RAMNEEK AHLUWALIA
CHIEF EXECUTIVE OFFICER



1.1 INTRODUCTION

The impact of the COVID-19 pandemic on the post-school education and training sector tested Higher Health's capacity to the limit. The organisation rose to the challenge and, during the period April 2020 to March 2021, became a significant role-player in the national pandemic response. Higher Health grew immensely through this intense period of collaboration aimed at protecting students and employees in the sector. We felt that our contribution was essential and, consequently, that the case for building strong health programmes within post-school education and training institutions has gathered substantial support.

The pandemic was a watershed that redefined life at so many levels – social, economic, cultural, political, technological and psychological. Its impact has mainly been destructive and our sector has not been

spared the loss of life, the burden of illness, the loss of income and the derailing of careers. Every time we pause to take stock – as we do in writing this report – we reckon with this terrible toll and the accumulation of pain that still surrounds us.

But, as with all major crises, the COVID-19 pandemic also elicited a flood of responses that were admirable, purposeful and effective – quite often heroic and occasionally unbelievable. Higher Health was proud to find a useful role for itself within the national mobilisation against COVID-19, working in close collaboration with the Ministry of Higher Education, Science and Technology and the Department of Higher Education and Training (DHET), universities and colleges countrywide, the Department of Health and health experts who generously shared their expertise.

1.2 ELEMENTS OF OUR COVID-19 RESPONSE

Our efforts to combat the pandemic centred on the following:

- Developing comprehensive **guidelines and protocols** for managing COVID-19 in the post-school education and training sector. These included procedures for screening, testing and self-isolation of students and staff, routine cleaning, disinfection after an incident of COVID-19, responding to confirmed cases of COVID-19, recruiting and training of screening volunteers, dealing with COVID-associated gender-based violence (GBV) and mental health issues, invigilation of exams, and management of isolation and quarantine facilities in campus residential accommodation.
- Providing **training** on the guidelines and protocols to nearly **50 000 staff members and student volunteers** at universities, technical and vocational education and training (TVET) colleges and community colleges. This number included a corps of about **14 900 screening volunteers** from the student population who ran screening stations at entry points to campuses, lecture halls and residences.
- **Initiating a mental health and GBV support service for students** that could be accessed by toll-free phone 24/7 and was literally at hand in times of psychological crisis. It assisted an average of 850 students a month.
- Introducing a fleet of 10 well-equipped **mobile clinics** to undertake COVID-19 screening, testing and contact-tracing in rural districts with substantial student populations. These mobile clinics also assisted during cluster outbreaks on campuses.
- Developing and marketing a multilingual **mobile application, HealthCheck** (available on



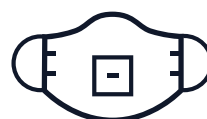
USSD, Whats App and the web) for students and staff to self-screen, determine their COVID-19 risk and take appropriate action. It effectively issued daily campus “passports” to the majority of users, simplified entry checks and avoided risky congestion at points of entry to campuses.

- Creating and distributing **educational materials**. More than two million leaflets and 50 000 posters were printed and distributed. Mass use of the mobile phone short messaging system (SMS) reached 680 000 students multiple times.
- Leveraging our established Future Beats programme – a partnership with selected **campus and community radio stations** – to reach up to a million listeners on and around campuses with key COVID-19 information.
- Conducting an **online survey** in partnership with the Human Sciences Research Council (HSRC) to understand students’ experiences of the initial COVID-19 lockdown period and appreciate the impact on their wellbeing. More than 13 000 young people responded.
- Assisting institutions to **contain cluster outbreaks** among staff and students. There were 11 such outbreaks at universities and four at TVET colleges. Higher Health created COVID-19 cluster investigation and response teams with a range of organisations, including provincial health departments, the National Institute for Communicable Diseases (NICD) and the National Health Laboratory Service (NHLS).

The above activities contributed to national COVID-19 objectives of lowering transmission, moderating pressure on the healthcare system, and reducing illness and lives lost. There were also clear benefits for the post-school education and training sector in terms of establishing and maintaining campuses as COVID-compliant work spaces and environments for contact learning and teaching. For many students, especially in the TVET sub-sector, this was a make-or-break development in terms of completing the tertiary qualification they had dreamed of.

There were also unique benefits for Higher Health and its project to advance health and wellness across the post-school education sector:

- The COVID-19 response gave Higher Health an opportunity to include community education and training (CET) colleges and some private institutions within the ambit of our support programmes. This was a step towards serving the sector comprehensively.
- It underscored the value of our investment in human infrastructure for health programme delivery on university and TVET college campuses. Higher Health was only able to step up and help in this time of national crisis because we had an extensive network of campus volunteers and on-the-ground staff already in place. We could mobilise and redirect them to combat the most urgent health threat facing the country and the sector.
- It helped form new partnerships and deepen existing relationships. Collaboration in times of emergency forges unique links between organisations and the trust that is established is a great asset. We feel that Higher Health has gained in terms of connectedness and reputation – and that we can put this to use in future in the interests of the sector we serve.
- It compelled us to become more innovative, like countless organisations around the world, and to leverage the power of digital technology to protect and promote student health. Digital communication and engagement are an excellent fit for our sector and we will continue to develop our ability to use technology, applying it to our ongoing programme of work outlined below.



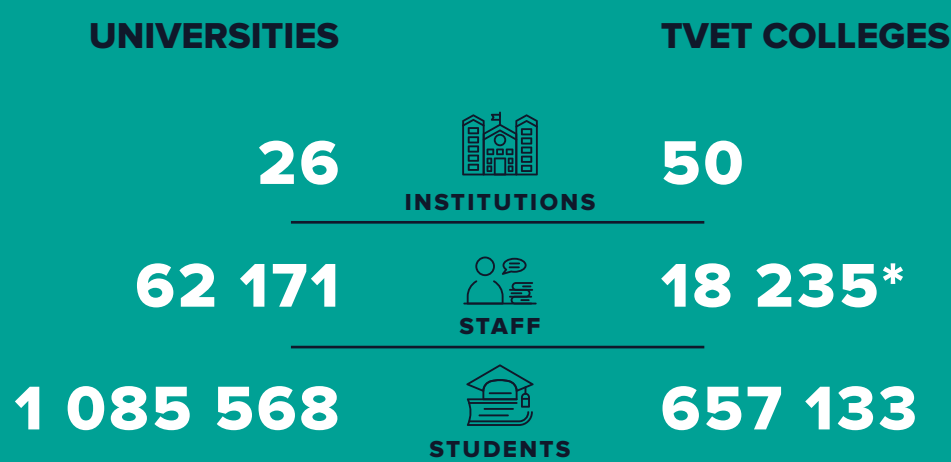
1.3 SUSTAINING OUR HEALTH VISION: NO PANDEMIC LEFT BEHIND

While prioritising the pandemic response, Higher Health made every effort during 2020/21 to ensure that its established programme of youth-centred health interventions remained on track. Many of Higher Health's focus areas – such as sexual and reproductive health (SRH), mental health, HIV and TB, and GBV – relate to diseases and health issues that are also of pandemic magnitude. Neglecting them was not an option. The following results indicate our ability to sustain these programmes:

- More than 1 000 student wellness days were held, many under socially distanced conditions.
- More than 100 000 students attended Second Curriculum “classroom” sessions (some were virtual) on GBV and mental health.
- Nearly 50 000 students participated in self-administered risk assessment for vulnerability to GBV and mental health problems. Where necessary, they were linked to appropriate services.
- The newly established toll-free mental health line was utilised by more than 7 000 students in the first eight months of operation.
- Nearly 120 000 students participated in the Higher Health Second Curriculum by attending virtual or face-to-face “classroom” sessions on HIV, STIs and TB.
- More than 80 000 students and staff opted for HIV testing and similar numbers for screening for TB and STIs.
- Nearly 44 000 contraceptive products and 5.7 million condoms were made available on campuses.

The numbers show that Higher Health and its flagship programme, First Things First, maintained a strong heartbeat during this period, despite limitations on students inhabiting their campuses and mixing freely. Major developments in our established programmes are described later in my overview.

Figure 1: Overview of the sector we serve.



Source: Statistics on post-school education and training in South Africa 2018. DHET, 2020. *Figure from TVET sub-sector report for 2019/20 sector skills programme. ETDP SETA and DHET

1.4 KEY SUCCESS FACTORS

Reflecting on 2020/21 it seems to me there were three critical approaches that enabled Higher Health to work productively through this period of crisis.

The first was **maximising the assets** we already had – the health and wellness model we had developed over the years, our conviction that students should be health activists rather than passive beneficiaries, our network of skilled, responsible staff members, and improved access to on-campus health services. These basic features of Higher Health stood the organisation in good stead and enabled us to scale-up our COVID-19 response rapidly as well as sustain other important health initiatives.

The second was **joining the technology revolution** when circumstances demanded this. In 2020, Higher Health incorporated digital and mobile technology not only as a communication tool but as an instrument for health screening and linkage to care. Arguably this was a step the organisation should have taken even before COVID-19 necessitated it. However, the urgency of the pandemic environment and the restriction of face-to-face contact propelled us into digital innovation.

The benefits for the community of students and staff we serve have been self-evident. We want to assure them that digital will be part of our thinking well into the future.

The third critical success factor was **partnerships**. Now partnerships are in the DNA of Higher Health and this was nothing new. However, we were forced to scale up our partnerships and build new relationships while in crisis mode. COVID-19 was an unknown quantity and outbreak management requires a particular approach that Higher Health had not previously been required to implement. We had to seek out expert knowledge and learn a lot very quickly in order to assist our sector. We are deeply indebted to the virologists and public health experts who taught us as they worked alongside us. We also owe much to our established partners who pragmatically adapted the terms of our collaboration to take account of the extraordinary circumstances we all faced. A third category of invaluable partners were those that provided additional resources to enable Higher Health to play its part more effectively – for example, by funding personal protective equipment (PPE) and providing a fleet of mobile clinics.

1.5 MAXIMISING OUR SYSTEMS AND RESOURCES

Higher Health was able to coordinate the sector's COVID-19 response and sustain its ongoing health and wellness programme by utilising the inherent strengths of the model that underpins all its work, by stretching the resources mobilised for a few critically needed new services, and – towards the end of the reporting period – increasing efficiency by blending on-campus COVID-19 activities with those targeting other health areas.

LEVERAGING OUR EXISTING MODEL

Higher Health has a clear model that guides its programme of activities on campuses across the country. Important features of this model (which is explained in more detail on [page 22](#)) are:

- It develops *health literacy* through structured educational sessions – constituting the Second Curriculum – and informal communication activities, led by peer educators and utilising mass communication methods. Our aim is not only to build knowledge, but to stimulate self-awareness, develop self-esteem and resilience, and cultivate respect in relationships.
- It bridges the gap between acquiring health knowledge and putting this knowledge to use. Students are encouraged to take simple actions that strengthen *ownership of their own wellbeing*. For example, they are given opportunities to complete a self-assessment of their individual risk in relation to HIV, TB and sexual health and in relation to mental health and GBV. There are also opportunities to be screened for TB, STIs, diabetes and high blood pressures and test for HIV. More recently, Higher Health has added a crisis helpline for psychosocial support to further enable health-seeking actions.
- It prioritises convenient *access to appropriate health services*. Over the years, Higher Health has worked closely with existing campus health clinics to align services with students' needs and we have collaborated with public health authorities to establish clinics on TVET college campuses, utilising health workers seconded

from the health district. In 2020, we went a step further and acquired and staffed 10 mobile clinics which service outlying campuses that were not included in earlier expansion efforts.

This approach has achieved a level of general health awareness and health competency on campuses.

In order to deliver this model, Higher Health has facilitated the *development of human infrastructure*, both in the form of students who volunteer each year as peer-to-peer educators and in the form of staff members on campus who run clinics, organise health and wellness days, and mentor peer educators. There is an annual cycle of recruiting and training peer educators in which Higher Health's campus health and wellness coordinators CHCWs and the institution's own staff play a vital role.

This model proved invaluable when COVID-19 posed an unprecedented challenge to the sector.

Higher Health knew it was possible to train large numbers of lay people to play a responsible role in the delivery of preventive health programmes and we had the human resources to undertake this training. The preparation of all categories of staff at colleges and universities for their role in the safe reopening of campuses was a daunting task – but not an impossible one. The training capacity within the Higher Health system was mobilised and it delivered.

With our experience in involving students in the delivery of health programmes, we were confident there would be a positive response to a call for screening volunteers and that they would take their duties seriously. This insight, too, proved correct.

There was mass communication and health promotion capacity on campus thanks to Higher Health's peer-to-peer methods and its longstanding collaboration with campus and community radio stations through its Future Beats initiative. (See [page 43](#) for detail.)



Not only were our systems capable of adaptation for the COVID-19 response but – equally importantly – they continued to provide essential health services to students alongside the pandemic response. Students needs for psychosocial support, SRH services, and prevention and treatment of HIV and TB did not disappear with the arrival of a more urgent threat. Arguably, their need for some of these services only increased.

To some degree, Higher Health was able to offer health promotion activities remotely. But our main contribution was the rapid re-activation of our health programmes as campuses swung back into life. The figures cited at the beginning of my overview are testimony to the resilience and commitment of campus health programme providers at all levels – from clinic managers to student volunteers and CHWCs.

The uptake of various health programmes – from the Second Curriculum and self-screening, to the supply of condoms and contraceptives, and the organisations of health and wellness days – is reported on [pages 23 to 44](#).

EXTENDING OUR SERVICE OFFERING

We are proud to say it was not business as usual in the second half of 2020 but *better* than usual because of two important additions to the programme, namely:

- The **24-hour helpline** offering psychosocial support to students experiencing a personal crisis.
- The launch of a fleet of **mobile clinics** to provide a range of essential health services to campuses that lacked dedicated student health services.

Mental health has been a priority of Higher Health for the last few years. We have been alive to the extraordinary stresses of student life and informed by evidence that mental health problems often emerge in the age group most students fall into. Until the current reporting period our interventions had focused on creating awareness about mental health, encouraging students to resist the stigma that surrounds mental illness, and building some capacity in campus health clinics to assist students and refer them for expert care.

The general underdevelopment of mental health services in South Africa – particularly in the public sector – and the under-supply and lack of diversity among clinical psychologists and psychiatrists has placed constraints on Higher Health's options in this area.

But the COVID-19 pandemic and resultant lockdown measures introduced a whole new set of stresses. Many households were in extreme financial distress, futures were uncertain, sudden loss of life was evident all around, and people were confined to their homes. Organisations working in relevant fields began to report a rise in psychosocial problems and a higher incidence of GBV.

Higher Health surmised that students would not be exempt from this trend – especially as their normal studies had been disrupted and many lacked satisfactory on-line study options. This was later confirmed by a survey we conducted in partnership with the HSRC which found that 65% of 13 119 students who participated had experienced some symptoms of psychosocial distress. In addition, 21% reported being subjected to emotional, physical or sexual violence and one in four of these said incidents of violence had become more frequent under lockdown conditions.

The unique circumstances spurred Higher Health to act with urgency: we partnered with the South African Anxiety and Depression Group (SADAG) to set up a 24-hour toll-free helpline for students in need of crisis counselling and support. Details of this service and the pattern of use are reported more





fully on [page 34](#). Suffice to say that the need for this service was real and substantial. Nearly seven out of 10 callers were young men, and we are encouraged by their willingness to seek help in a society where men tend to avoid dealing with physical and mental health issues. We are also encouraged to see that students who have been subjected to GBV are using the helpline as a first step to access more comprehensive assistance, as are students struggling with substance abuse.

The Department of Science and Technology made available 10 fully equipped mobile clinics which were launched by Minister of Higher Education, Science and Technology Dr Blade Nzimande and enabled Higher Health to improve access to a package of essential services for students at smaller and more remote TVET campuses.

**OUT OF
13 119 STUDENTS**

65%

**EXPERIENCED SOME SYMPTOMS OF
PSYCHOSOCIAL DISTRESS.**

21%

**REPORTED BEING SUBJECTED TO
EMOTIONAL, PHYSICAL OR SEXUAL
VIOLENCE**

ADAPTING PRACTICES AND MERGING PROGRAMMES

While Higher Health's basic model provided tools for the post-education and training sector to respond to the COVID-19 pandemic and attend to the wellbeing of students, some important modifications had to be made in how we delivered the model.

Our health interventions were adapted to become compliant with prevailing regulations under the national state of disaster. This meant ensuring social distancing during all health promotion and screening activities. For example, the classroom-based Second Curriculum programme had to reduce by half the number of participants in each session and double the number of sessions held.

We also increased our mass communication efforts in order to compensate for reduced face-to-face information sharing. The production of materials such as leaflets and posters was stepped up and the use of mobile phone short messaging and social media platforms greatly increased. Details of this appear on [pages 41 to 44](#).

Over time, we realised that it was possible to integrate COVID-related interventions with our long-standing programmes. COVID screening points saw every student present on campus at least once a day and this presented an opportunity to deliver other preventive health activities, such as condom distribution and screening for HIV, TB and NCDs. We even began to modify First Things First wellness days so that they were COVID-compliant and position these interventions close to COVID screening points. In short, the effort we had invested in making campuses COVID-safe, began to deliver returns to the broader health programme at many universities and colleges.

VENTURING INTO mHEALTH

Traditionally Higher Health has developed health interventions that rely on face-to-face communication, largely in keeping with the contact learning and teaching culture of most institutions in the sector. We have enjoyed using social media platforms but have not relied on them as a central pillar of programme delivery. However, we have been following the unfolding of mHealth – that is, the use of mobile-based digital technology for a variety of health purposes – and saw this as an option for the future.

COVID-19 persuaded us to venture into the unfamiliar earlier than expected when institutions



in the post-school education and training sector faced the mammoth task of screening every person entering campus on a daily basis. For large institutions, this meant screening literally thousands of students and staff in the space of an hour or two. It presented the risks of large crowds forming, social distancing being compromised, and learning and teaching suffering from long delays.

The solution lay in the development of a digital self-screening application that could be used in conjunction with manual screening and speed up the process of clearance to enter campus or a particular residence or lecture hall. Higher Health developed this in conjunction with the national Department of Health (NDoH) and the Bill and Melinda Gates Foundation. The project has been sustained through support from the National

Skills Fund (NSF), European Union (EU), the Health and Welfare SETA and various private sector organisations.

Students and staff were advised to use the HealthCheck app each day before they left home for campus. Students and staff who received a HealthCheck “passport” proceeded to campus confident of admission. Those who appeared to be ill or had been exposed to someone with COVID-19 were declined clearance by the app. They stayed away from their institutions and the risk of transmission in a screening queue was eliminated. The added advantage was students who were categorised as moderate to high risk received information to guide their next steps – for example, going for a COVID test and/or self-isolating and/or consulting a health professional.

Figure 2: How the HealthCheck app simplified campus screening



HealthCheck attracted 1.6 million unique users – almost equal to the total number of students enrolled at universities and TVET colleges – and was used 10.3 million times in the course of just eight months.

The acceptability of HealthCheck and the way in which it complemented manual screening activities suggest that Higher Health should explore converting some of its other paper-based self-assessment tools to digital format. This could achieve greater efficiency, reach more students, and free up precious human resources for more complex activities.

In developing HealthCheck we were mindful of the fact that a digital divide exists even within the world of tertiary education. When at home, the HSRC survey revealed, many students have little access to data unless they receive an allocation from their university or college. Therefore, we decided our app should be accessible via our website, WhatsApp or USSD, which does not require data or a smart phone. It was striking that 57% HealthCheck users chose the USSD option.

1.6 FORMING AND CEMENTING PARTNERSHIPS

Solid partnerships, nurtured over the years, have been a major factor in Higher Health's progress and the strengthening of campus health and wellness programmes. Many organisations – public, private and non-profit – have contributed financial and other resources to our sectoral programme of action. Of equal importance is the confidence they have shown in our ability to deliver.

It was important to preserve the trust of these partners as COVID-19 displaced and disrupted the programmes they had supported. We did this by communicating honestly with funders about the constraints we were experiencing and by sustaining programmes as best we could during abnormal circumstances.

We appreciate the flexibility of all our funders, who have been willing to adjust grant time scales and targets. We feel confident we will be able to deliver the expected value within the adjusted deadlines.

In the heat of the COVID-19 mobilisation, Higher Health also forged a set of new partnerships with organisations we had scarcely encountered in the past. The generosity of individuals in these structures has been remarkable – especially their willingness to share knowledge – and this is an enduring positive to be taken from this devastating health emergency.

The factor that unlocked this goodwill was the trust displayed by Minister Nzimande in delegating to Higher Health many key responsibilities for equipping the sector to withstand the COVID blast. This opened important doors to assistance and collaboration.

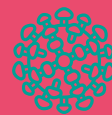
We felt privileged to gain access to a clan of epidemiologists, virologists and infection control specialists who enabled Higher Health to develop the sector's COVID-19 guideline and associated protocols, and produce training materials to ensure these protocols were implemented consistently. This technical assistance was invaluable. In addition, a number of organisations stepped forward to provide financing for the COVID-19 response, including: the DHET, the NSF, the HWSETA, the EU, the Global Fund, the World Bank, the DSI, and Old Mutual. These covered the costs of: PPE, IEC materials, the development and running of HealthCheck, communication costs and staffing of the GBV and mental health helpline, salaries of psychologists and nurses, and costs of the mobile clinics.

TECHNICAL EXPERTISE

From time to time, Higher Health has created advisory structures to guide the development of its programmes. The vital role that experts from the World Health Organization (WHO), NICD and various universities played in building Higher Health's grasp of the COVID-19 challenge spurred us to establish a number of task teams to continue to advise us in years ahead. (For details of these teams see [page 25](#).)

We are grateful to the individuals who have agreed to serve on these teams and look forward to learning from their understanding of the science and best practice in their specialist areas.





1.7 FINANCIAL SUMMARY

Higher Health experienced a substantial increase in income from grants in response to the needs that were magnified by the COVID-19 situation and dispersed almost 95% of its total income.

The biggest investment was in our staff, with personnel costs accounting for 40% of expenditure (R29m). Another significant item of expenditure was production of promotional and educational materials (R10m).

Table 1: Summary of income and expenditure for 2020/2021

Item	2020/2021	2019/2020	Year-on-year change
Income			
Revenue – grants	R 72.46m	R 42.31m	71%
Other income	R 5.55m	R 2.94m	88%
Total income	R 78.01m	R 45.25m	72%
Expenditure			
Operational expenses	R 69.60m	R 40.42m	72%
Admin expenses	R 4.27m	R 1.79m	138%
Finance costs	R 0.35m	R 0.13m	169%
Total expenditure	R 74.22m	R 42.34m	75%
Surplus for year	R 3.79m	R 2.91m	

1.8 LOOKING FORWARD



In the coming year, Higher Health looks forward to further strengthening our health and wellness programme and incorporating the COVID-19 response as a regular component of this programme. It is clear we will be living with this roller coaster pandemic for some years and need to mainstream relevant precautions and services.

There will, of course, be a major COVID-19 vaccination campaign in the second half of 2021, with on-campus sites where feasible and a strong promotional component. Higher Health has become a trusted brand on campus and we intend to use our credibility, our network of peer educators and our communication platforms to overcome vaccine hesitancy and boost the uptake of vaccination.

We also intend to focus more consistently on building our programme to prevent GBV. A large amount of preparatory work was done during the 2020/21 year, drafting and generating support for protocols that will translate the national policy framework into practical interventions. Among these is a document providing guidance to institutions on measures to be taken against sexual harassment of all types – among employees, between staff and students, and among students. A zero-tolerance approach to GBV must include all forms of sexual coercion, including those that are commonly excused as “non-violent”. We must rid our campuses of the sex for grades scourge. We must speak through words and unambiguous actions to create an environment where all women and all LGBTQI students feel secure and are secure.



1.9 ACKNOWLEDGEMENTS

Higher Health was truly humbled by the trust our partners placed in us to play a leading role in the sector during this time of crisis. We were also never short of capable assistance. Wherever support was needed – whether in the form of technical advice, funding or willing cooperation – our established partners rose to the occasion and new partners emerged.

It is impossible to acknowledge every contribution individually, but I would like to mention some of our major supporters.

We are grateful to Minister of Higher Education, Science and Technology Dr Nzimande for trusting Higher Health to play a key role in the sector's response to the pandemic and to Deputy Minister Buti Manamela – who chaired the sector's Ministerial Task Team on COVID-19 – for their unwavering support.

We would like to pay tribute to the vice-chancellors of universities and principals of TVET and CET colleges for the unified response they were able to mount and the commitment they showed to the safety of their staff and students.

The technical guidance of the NICD, NHLS and WHO was irreplaceable. Higher Health was a novice in terms of outbreak response when the pandemic reached this country, and we were totally reliant on the generous sharing of expertise by these entities, the DOH, provincial health departments and district health authorities. We can honestly say their help never flagged and they were at hand throughout the year as new challenges arose.

International funding organisations, United Nations agencies and local public and private sector funders unfailingly responded to Higher Health's COVID-19 funding requests and proposals. They were also understanding about the impact of COVID-19 on our ability to meet other programme targets. We have tried to honour the confidence these organisations have shown in our organisation by utilising resources to the best of our ability under trying conditions.

Non-governmental and community-based organisations continued to play an invaluable role in sustaining First Things First campaigns on campuses. It is indeed important to ensure the COVID-19 crisis does not undermine the work we have built up over the years.

My deepest gratitude goes to the people who constitute Higher Health: members of its governance, advisory and operational teams. I can only say that the past year tested our abilities as no other before it and that you rose to meet impossible challenges.

Firstly, to the Higher Health Board and particularly our chairperson, Prof Wim de Villiers: in times of pressure wise counsel is especially valuable and we had a Board that was truly vested in our ability to steer the higher education and training sector through this crisis.

Secondly, to chairs and members of the technical task teams that sprang into life in response to the ever-changing demands of the pandemic on our staff and student population. We borrowed heavily on your insights, experience, reputation and time to give Higher Health capacity we simply lacked within our established ranks. Once more – your readiness to help was unfailing.

Finally, to our managers and staff at Higher Health who answered an extraordinary call of duty without hesitation and sustained an enormous effort over the months as the pandemic reinvented itself: you acted selflessly and often put your own health and safety at risk because you recognised this is what the situation demanded. I am both proud and grateful for what we have achieved collectively.

**DR (PROF) RAMNEEK
AHLUWALIA**
CHIEF EXECUTIVE OFFICER

2 SECTION



TOT AN ORGANISATION IN MOTION



2.1 INTRODUCTION

Higher Health, as an independent non-profit organisation, celebrated its third birthday during the 2020/21 financial year. But the organisation emerged directly from an older initiative, the Higher Education and Training HIV/AIDS Programme (HEAIDS), established in 2000 by the then Department of Education as a programme of government implemented by Universities South Africa, (USAf, formerly known as Higher Education South Africa, or HESA).

The heritage of Higher Health is important: the relationships built under the banner of HEAIDS – with government, with universities and TVET colleges and their representative bodies, and with funding organisations – have been critical to the new organisation's growth and success.

At the same time, reinventing HEAIDS as a non-profit organisation and taking on the new name, Higher Health, has been immensely beneficial. Higher Health has gained the agility necessary to respond to this dynamic (and sometimes tumultuous) era. Perhaps counter-intuitively, the flexible nature of Higher Health has made it a more valuable partner to the DHET and to universities and colleges with their more protracted processes.

This section of the report deals with the structure of Higher Health, the focus of its work, and the methods it uses to pursue its goals, which are to:

- Enhance student health and wellness in order to increase the proportion of students who complete their studies and reduce the time it takes for students to graduate.
- Build the knowledge, understanding, skills and capacity of students in relation to general health and holistic wellness.
- Help produce graduates who are well-prepared for family and career responsibilities by developing their grasp of the importance of health in these contexts.
- Develop a campus environment that is conducive to health by nurturing a commitment to human rights as a fundamental aspect of student development and wellbeing.
- Improving organisational effectiveness and efficiency in order to reduce organisational costs and increase funding for health and wellness-related programmes.
- Strengthen and expand partnerships with higher education institutions, relevant government departments, funding and donor agencies, and other organisations active in our field.
- Strengthen and diversify our funding base, ensuring a variety of funding sources in order to secure operational sustainability.

2.2 HIGHER HEALTH STRUCTURE

The organisational design that enabled Higher Health to become an effective resource for universities and colleges as the COVID-19 crisis hit was built over the previous two years. The critical feature of this structure is that it has been created to complement and strengthen the considerable capacity present among staff and students of the institutions themselves.

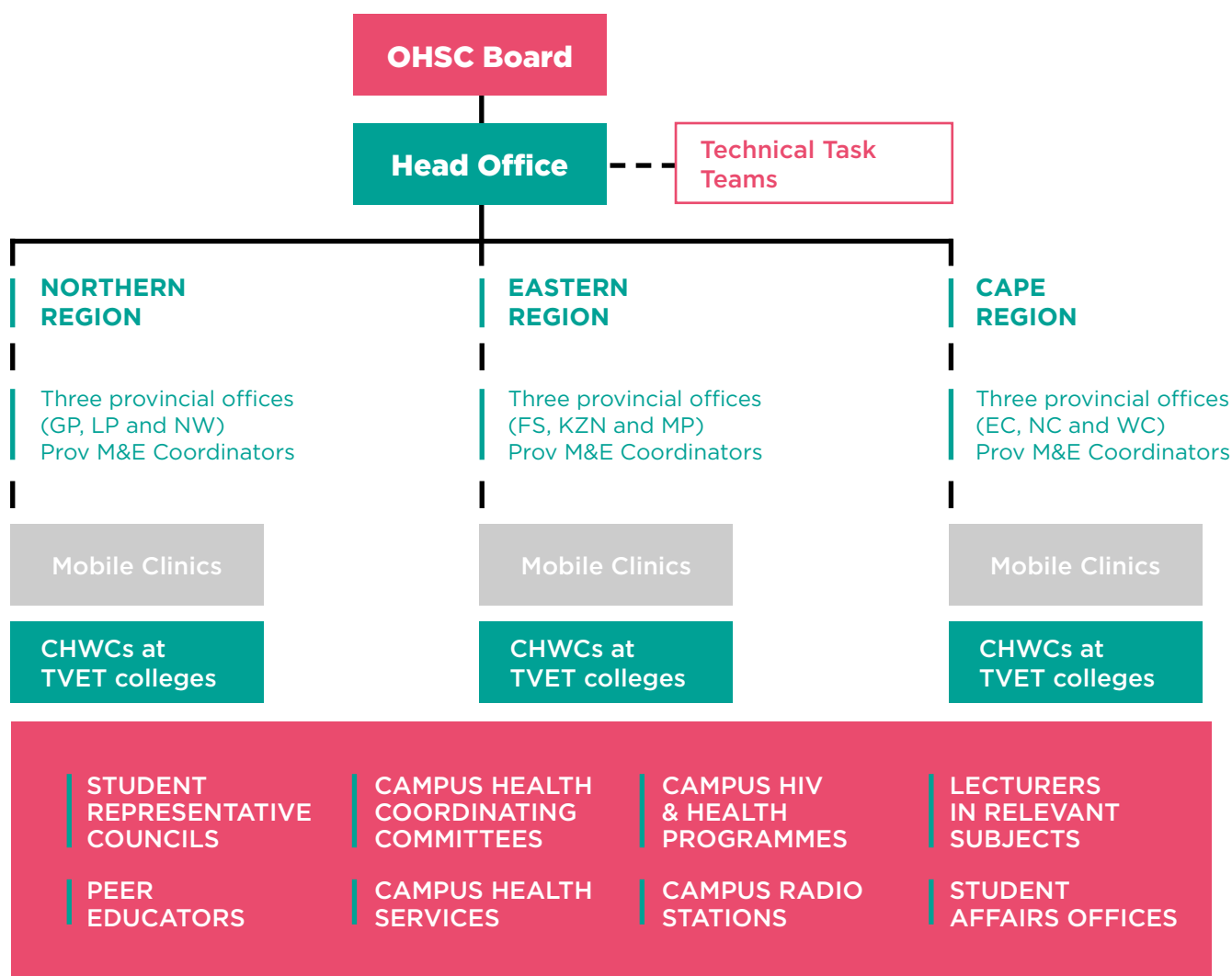
It has taken Higher Health time to understand fully what functions it should assume and what responsibilities should reside with the institutions constituting the sector. It is apparent that all institutions benefit from strategic guidance, capacity building, and provision of key resources.

In addition, many TVET colleges have less in-house capacity to deliver health programmes than universities and require more direct support. This Higher Health provides through:

- Campus health and wellness coordinators (CHWCs) who are based in TVET colleges but are accountable to Higher Health through provincial offices.
- Mobile clinics with appropriate staffing that service colleges in more remote areas which lack on-site clinic facilities.

Higher Health's structure and the institutional capacity it seeks to complement are depicted in **Figure 3**.

Figure 3: Complementary capacity: Higher Health's structure and institutional resources





The primary responsibilities of CHWCs are to assist TVET colleges to organise First Things First activations and wellness days, build the capacity of college staff and students to undertake health activities, and present educational sessions within the Second Curriculum. Their core skills are coordination and training – and these proved invaluable during the COVID-19 mobilisation.

The establishment of regional and provincial offices also benefits universities, because Higher Health is more accessible to them and better placed to support their efforts to strengthen their health programmes.

Over the years, Higher Health has advocated for the establishment of dedicated management structures for institutional health programmes and has developed sound working relationships with campus health services, student affairs offices, student representative councils, campus radio stations, lecturers in relevant subjects, and individual students serving as peer educators. We have mobilised resources and provided training to enable these structures to engage productively in our programme of student health, wellness and development. Our campus counterparts were another powerful resource for the sector to tap into when confronted with COVID-19.



2.3 HIGHER HEALTH FOCUS AREAS

While Higher Health adopts a broad primary care approach, it encourages universities and colleges to focus on health issues that most affect the student population. The specific health vulnerabilities of young people are well documented and Higher Health relies on this evidence to set priorities.

In the reporting year, Higher Health added COVID-19 to its focus areas once it became clear that universities and colleges would be living with waves of infection for some time to come.

Table 2: Health Health's focus areas and reasons for selecting them

FOCUS AREAS	WHY WE PRIORITISE THEM
Human immunodeficiency virus (HIV), tuberculosis (TB) and sexually transmitted infections (STIs) Prevention, early diagnosis, linkage to treatment and support systems	Incidence of HIV is declining in all age groups but is still highest in the 15 – 24 age group (Thembisa Model 4.3) Young women are three times more likely to acquire HIV than young men and are most vulnerable between 18 and 20 years (Thembisa Model 4.3) Sexual risk behaviours are highest in 15 – 24 age group but so is the use of prevention tools, such as condoms and medical male circumcision (HSRC 2017) The TB treatment gap is high for young people (DoH TB survey 2020)
Sexual and reproductive health (SRH) Contraception, antenatal care, prevention and early detection of relevant cancers	One-third of TVET students have been pregnant or made someone pregnant (HEAIDS 2014) and three out of four such pregnancies were unplanned Other studies – SADHS, HerStory and DREAMS evaluation – confirm this
Gender-based violence (GBV) Awareness and advocacy, policy development, care of survivors	Nearly one in three adolescent girls and young women in HerStory study reported experiencing GBV Police figures indicate 20% of women reporting rape are younger than 20 years (SAPS 2019)
Mental health Awareness, combating stigma, early access to services	Unnatural deaths (suicide, accidents and homicide) constitute nearly half of all deaths in young men (StatsSA) A university study has shown that 12% of students have symptoms of depression and 15% of anxiety
LGBTQI+ health Awareness, promotion of rights, appropriate services	Young LGBTQI+ people experience discrimination and even violence Many find health services ill-equipped to meet their needs
Alcohol and drug abuse Awareness, combating stigma, access to services	60% of TVET students regularly misuse alcohol (HEAIDS 2014) Binge drinking occurs among 20% of SA men aged 15–19 years (WHO 2018)
Disability Awareness, promotion of rights, access to services	People with disabilities experience discrimination and exclusion from health programmes and some campus facilities
COVID-19 Disease prevention and control	With their lecture rooms, libraries, canteens and residences, universities and colleges are complex congregate settings with high potential for the spread of airborne infection

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2.4 HIGHER HEALTH MODEL

The Higher Health approach to campus health and wellness has a strong preventive foundation complemented by access to appropriate, youth-friendly services. Prevention programmes seek to create awareness of health matters, promote health rights and combat stigma, provide access to relevant information through a variety of reinforcing channels, and enable students to assume responsibility for their health.

The aim is to address health risk factors rather than treat illness but, where this fails, Higher Health regards early diagnosis and treatment as a positive result.

Over recent years a major element of the Higher Health programme has been advocating for the establishment of appropriate health services on all

campuses, within convenient reach of students. Partnerships with provincial and district health services took us a long way down this road, but there were still campuses – typically remote and relatively small – without clinical care. In 2020/21 Higher Health, with generous donor support, was able to fill this gap by leasing and staffing 10 mobile clinics. (See [page 35](#) for detail.)

The Higher Health model is founded on years of campus-based work and is constantly evolving. The latest version is depicted in [Figure 4](#).

More detailed descriptions of these interventions, together with indicators of output in 2020/21, are set out in *Section 3: Performance of Core Programmes*.

Figure 4: The Higher Health model for student health and wellness



LEVEL 1 **KNOWLEDGE TRANSFER**

2021 target: 700 000 students

Second Curriculum classroom sessions
Peer-to-peer educational activities
Campus & community radio programme
Social & mass media
First Things First activations



LEVEL 2 **PRACTICAL LEARNING**

2021 target: 630 000 students

Risk self-assessment
Risk screening
Diagnostic testing
Use of toll-free crisis helpline



LEVEL 3 **LINKAGE TO SERVICES**

2021 target: 315 000 students

Campus health clinics
Higher Health mobile clinics
Referral to external health services
Mental health services
Linkage to care & support groups



PERFORMANCE OF CORE PROGRAMMES



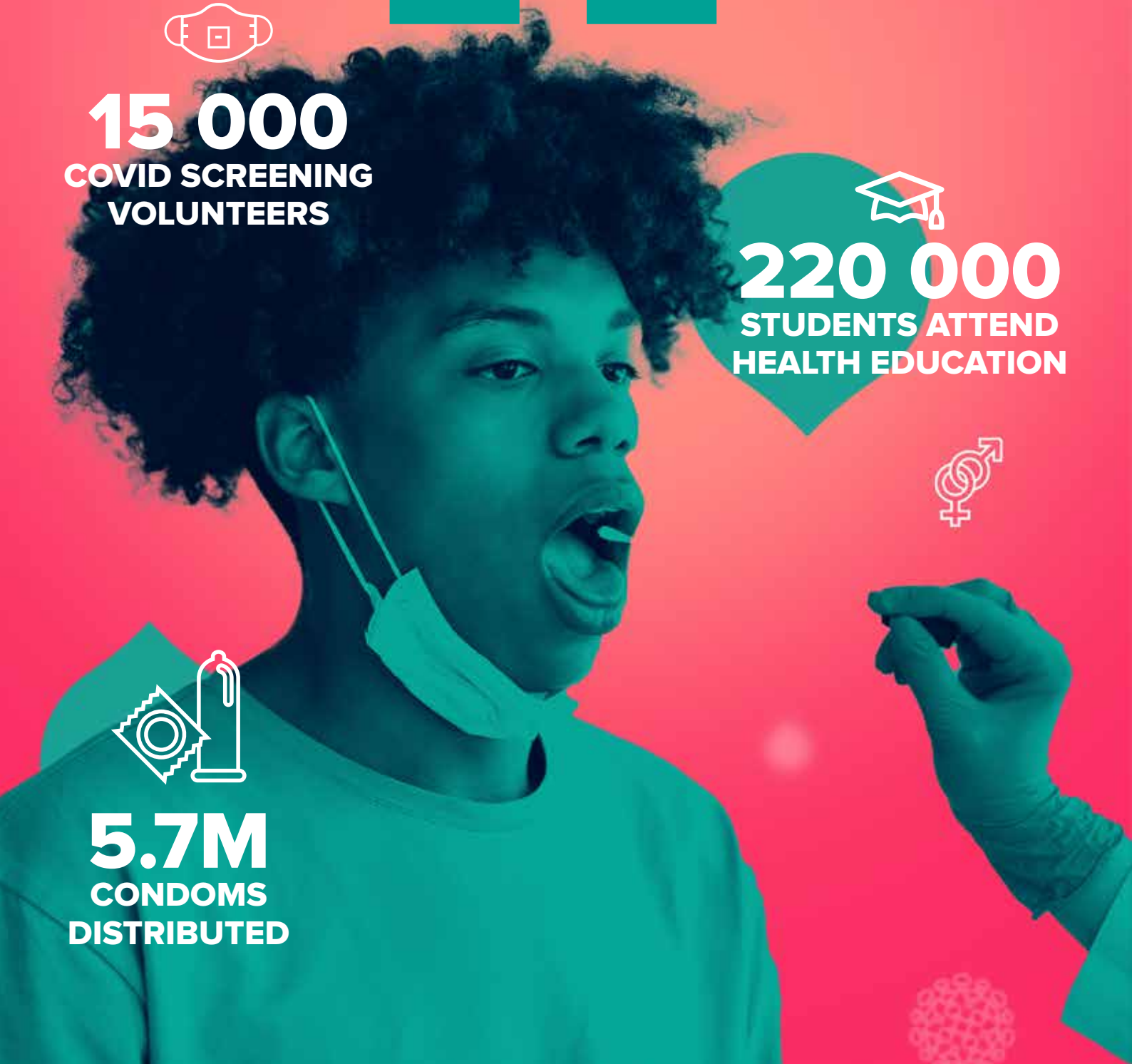
15 000
COVID SCREENING
VOLUNTEERS



220 000
STUDENTS ATTEND
HEALTH EDUCATION



5.7M
CONDOMS
DISTRIBUTED



3.1 INTRODUCTION

External circumstances directed Higher Health's energies for much of 2020/21: carefully laid plans were temporarily set aside to contribute to the national COVID-19 response. With access to some 2 million students and staff members, the post-school education and training sector clearly had a major contribution to make – and Higher Health was uniquely positioned to support the sector. Our intense involvement in many collaborative efforts was fruitful and rewarding.

Later in the year, once policies, procedures, tools and human infrastructure to contain COVID-19 were in place, Higher Health resumed its regular programme of interventions. Naturally, all activities had to be adapted for the COVID reality, using virtual platforms for engagement and reducing the size of face-to-face sessions to relatively safe numbers. We also found that solutions created in the heat of crisis to contain COVID-19 had prospects for longer and wider use in relation to our standing focus areas – such as HIV, TB and mental health.

3.2 COVID-19: THE CAMPUS CHALLENGE

The Minister of Higher Education, Science and Technology invited Higher Health to participate in the post-school sector's COVID-19 response. Under the leadership of the DHET, certain coordinating functions fell to Higher Health as the only national structure dedicated to student health and wellness. However, it must be stressed that every result described below was achieved through collaboration with organisations including – but not limited to – the DHET, NDoH, NICD, WHO, NHLS, provincial health departments and universities and colleges across the country.

3.2.1 DEVELOPING A COVID-19 GUIDELINE AND PROTOCOLS

The first project Higher Health became involved in, virtually as President Cyril Ramaphosa declared a national state of disaster in March 2020, was the development of a guideline to assist universities, TVET colleges and CET colleges to safeguard and provide support to their students and staff members. We worked alongside health experts, including virologists and outbreak management specialists, who were drawn from the WHO, NDoH, NICD and various universities, with the aim of providing authoritative, evidence-based guidance that would enable institutions to continue their essential functions of learning and teaching in a responsible manner.

For the first time, Higher Health served as a resource

not only to universities and TVET colleges, but also to nine CET colleges with thousands of centres in local communities.

The guideline laid the foundation for the sector's pandemic response and dealt with preventing the spread of infection, assisting individuals who had been infected with or exposed to COVID-19, managing the closure of institutions or parts of institutions, and the safe resumption of contact teaching and learning.

In addition to the guidelines, Higher Health oversaw the development of protocols to guide practical implementation on:

- Screening, testing, contact tracing, self-isolation and linkage to care.
- Routine cleaning of institutions for COVID-19 prevention.
- The recruitment and training of student screening volunteers, and their protection from infection.
- Management of mental health concerns and substance abuse in the context of COVID-19.
- Responding to confirmed COVID-19 cases at institutions within the sector.
- Managing cluster outbreaks of COVID-19 at universities and colleges.
- Invigilation of tests and examinations in the context of the COVID-19 pandemic.



3.2.2 FORMALISING THE ROLE OF EXPERT ADVISORS

For Higher Health, stepping into the arena of outbreak control was a journey into unknown territory and we relied heavily on the advice of experts in the field. Initially, this guidance was made available on an ad hoc basis, but we came to appreciate that the science on COVID-19 was rapidly developing and we would require teams of people with the expertise to interpret the evidence and help us formulate appropriate responses.

As a result, Higher Health constituted several technical task teams dedicated to different aspects of the COVID-19 response. They are:

- The Scientific Technical Task Team which includes representatives from the NDoH, WHO and NICD with expertise in virology, epidemiology and epidemic management. It is chaired by Prof Tim Tucker.
- The Mental Health Task Team, comprising psychologists and other mental health professionals, which has advised on responses to the mental health impacts of COVID-19 and also on broader programming. It is chaired by Prof Melvyn Freeman.
- The GBV Task Team, which is concerned both with specific vulnerabilities to GBV in the COVID-19 environment and with Higher Health's longer-term response. It is chaired by Prof Puleng LenkaBula.
- The Communication Task Team, the largest structure, with representatives from the DHET, universities, the National Student Financial Aid Scheme (NSFAS), NSF and student structures. It is chaired by Prof Ahluwalia.

3.2.3 BUILDING CAPACITY FOR EFFECTIVE IMPLEMENTATION

IMPLEMENTATION STRUCTURES

University and college level:

Management Task Team on

COVID-19 and its sub-teams.

Campus level: COVID-19 Steering Committee. Higher Health sits on every Steering Committee.

Higher Health assumed responsibility for assisting universities and colleges to implement the guidelines and protocols correctly. This took the form of building understanding of the required action among various categories of staff and student volunteers, and providing ongoing guidance and mentorship.

Together with the NICD and NDoH – and later the Scientific Technical Task Team – Higher Health delivered a programme of online and face-to-face capacity building sessions. Participants ranged from institutional management and academics to frontline staff, such as cleaners and security personnel, and student screening volunteers and peer educators. Content and methods of teaching were tailored to the requirements of participants.

In all, 49 645 individuals from the campuses of universities, TVET colleges and community colleges participated in this programme of capacity building between April 2020 and March 2021.

Remarkably, 80% of participants attended face-to-face training on university and TVET college campuses. Higher Health provincial staff and TVET-based CHWCs, exercising care to prevent infection, put in an extraordinary effort to achieve this training, driven by their passion to open the doors of lecture halls, libraries and residences safely.

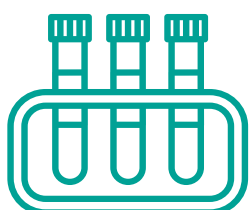
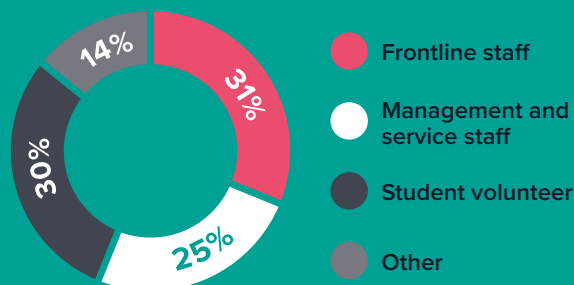




Figure 5: Capacity building for COVID-19 management on campuses

PROFILE OF PARTICIPANTS



PACE OF CAPACITY BUILDING APR 2020 TO MAR 2021



Note: Service staff are mainly student support staff. Frontline staff include nurses, residence officers, food service workers, cleaners and security staff.

3.2 4 FILLING THE PPE GAP

Higher Health ensured that its own staff had appropriate personal protective equipment (PPE) as they worked in the field continuously during lockdown Level 5 and subsequently, as restrictions eased.

In addition, the organisation procured and distributed limited supplies of PPE for use by frontline staff,

campus health service workers and student screening volunteers as universities and colleges first began to reopen for contact teaching and learning. The largest share went to TVET colleges where students had been less able to study remotely and the return to class teaching was imperative.

Table 3: COVID-19 PPE procured and distributed

Item	Number procured	Number distributed
Cloth masks	80 000	75 650
Surgical masks	15 000	14 050
N95 masks	1 000	940
Face shields	7 500	7 150
Non-sterile gloves	7 500	7 200
Sterile gloves	1 000	524
Body covers	7 500	4 260
Hand sanitiser	15 000	14 101





3.2.5 MEETING THE CHALLENGE OF MASS SCREENING

As the prospect of resuming on-campus learning and teaching came into focus around early May, the DHET and Higher Health confronted the challenge of screening everyone seeking access to campuses. An effective system was needed to avoid crowds of students and staff of unknown COVID status congregating at main entrances and exacerbating delays to enter the premises, which would disrupt learning and teaching.

The solution consisted of two main elements:

- Recruiting, training and protecting an “army” of student volunteers dedicated to manual screening at the main entrances to institutions and at entry points to lecture halls and residences.
- A mobile phone application designed to simplify the task of screening volunteers, speed up the process, and reduce the number of at-risk or infected individuals presenting for manual screening.

SCREENING SMARTER

The mobile phone application, known as HealthCheck, enabled students and staff to self-screen daily before leaving home and only proceed to campus once the “HealthCheck clearance granted” message was displayed. In cases where self-screening generated a “HealthCheck clearance denied” message, the individual received guidance about COVID-19 testing, self-isolation and accessing medical care, as appropriate.

Five language options were available – isiZulu, isiXhosa, Sesotho, Afrikaans and English – and the app could be accessed on the Higher Health website, by adding the number to WhatsApp contacts or by using a dedicated USSD line.

Minister Nzimande launched HealthCheck in June 2020 and within six months 1.6 million unique users had registered on the app. By March 2021 more than 10 million daily “passports” had been issued.

The rapid and extensive uptake was due to consultation with stakeholders and use of Higher Health’s established campus communication systems to market the app.

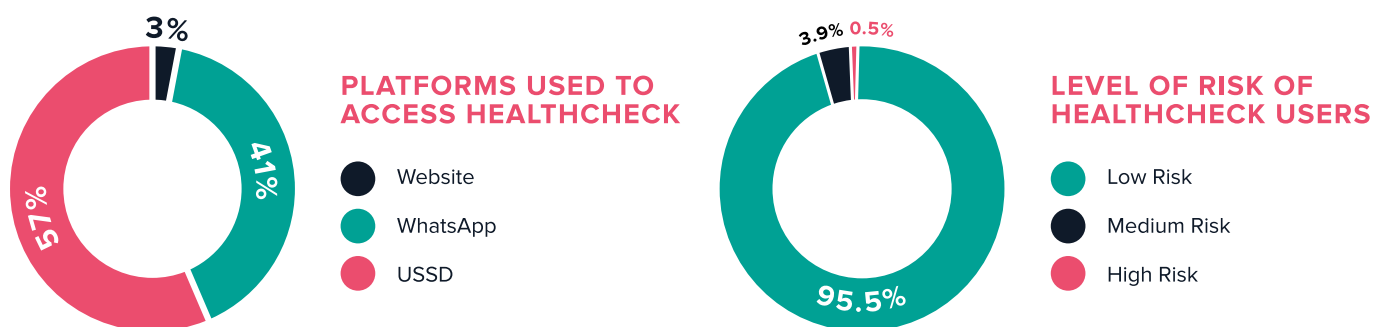
- Agreements on its use were reached with management and trade unions at universities, TVET colleges, community colleges and private colleges.
- There were meetings with student leadership and a wider circle of managers to secure buy-in.
- An SMS campaign to promote HealthCheck reached more than a million students.
- Peer educators on campuses drove awareness and demand.
- Campus and community radio stations participating in Higher Health’s Future Beats programme publicised the smart screening option.

An added benefit of HealthCheck was its automatic generation of basic demographic data on its users and the risk profile of users. Data could be disaggregated by institution, providing management with information on the possible emergence of COVID-19 hotspots. Unsurprisingly, given established health service consumption patterns, women surpassed men in taking advantage of this self-screening technology.

However, the true value of this venture into digital healthcare may be longer term. The urgency of the COVID crisis forced Higher Health to be more innovative and, having ventured to use technology to enhance our offering to students and staff, we are keen to apply it to our long-standing focus areas of SRH, HIV, TB, mental health and GBV prevention and care.



Figure 6: HealthCheck broken down



THE HUMAN TOUCH

Manual screening at the entry points to campuses, residences and lecture halls was performed to a large extent by trained student volunteers – 14 903 were in operation between April 2020 and March 2021 – who also provided information and assisted staff and students to register and use the HealthCheck facility. The volunteers helped to educate students and staff, neutralise myths, reduce stigma and refer people to testing facilities or other appropriate services.

TVET colleges fielded by far the majority of screening volunteers – 85% of the total – with universities accounting for the balance. Some volunteers already had health promotion experience as peer-to-peer educators within the Higher Health model. As COVID-19 screening settled into a routine, Higher Health recognised the potential to maximise the process for other health education and screening activities. For example, modified First Things First health and wellness activations were relocated to entry points and offered in COVID-safe fashion.

3.2.6 CLUES TO THE STUDENT EXPERIENCE OF LOCKDOWN

Higher Health and the HSRC joined hands to conduct a survey at the peak of the first COVID-19 wave to gain insight into students' experiences of the lockdown which had diverted their studies. It was an online survey, conducted on a data-free platform, and 13 119 young people enrolled at post-school institutions responded. Although the sample was not random, it was a good reflection of the overall student population.

The “snapshot” of student life from July to September 2020 is a pattern of light and dark: not all students experienced the situation in a similar way. Support structures worked well for some students and less well for others. Stresses in one area appeared to be counter-balanced by support in another. What is clear, however, is that most institutions had gaps in the learning support they offered students. TVET colleges as a group were least equipped to ensure that students could continue to learn remotely. Some of the findings are summarised in the table on [page 30](#).

3.2.7 EXTINGUISHING CLUSTER OUTBREAKS

The first cluster outbreaks of COVID-19 in the sector occurred in the Eastern Cape in October 2020 and involved students from university and TVET college campuses in Buffalo City. Higher Health coordinated a multidisciplinary team with relevant experience and skills to assist the affected institutions assess the situation and speedily implement a series of actions to cut the chain of transmission and contain the outbreak.

Once information had been gathered to identify the source of the outbreak and probable paths of transmission, interventions to bring the outbreak under control were instituted. These included: contact tracing and testing of those considered to be at risk, with rapid turnaround on results; identifying appropriate isolation and quarantine facilities for those exposed and infected; communication across the campus community with the aim of preventing panic and combatting



stigmatisation of those affected; and appropriate sanitising of selected areas.

Outbreak teams typically comprised representatives of WHO, NDoH, NICD and Higher Health, provincial and/or district health officials, local staff of the NHLS, and relevant managers from the institution(s) concerned.

CLUSTER OUTBREAKS

Walter Sisulu University
University of Fort Hare
Rhodes University
University of KwaZulu-Natal
Mangosuthu University of Technology
University of Zululand
North West University
University of Venda
University of Pretoria
Ikhala TVET College
King Hintsa TVET College
King Sabata TVET College
Ingwe TVET College

Table 4: Social impact of COVID-19 pandemic on post-school students

TOPIC	FINDINGS
Place of residence during lockdown	40.1% went home due to the COVID-19 lockdown 37.5% normally stayed at home and remained there Only 6.7% remained in residences on campus
Food security	79.3% relied wholly or partly on family for food 10% relied on food parcels or donations Another 14.9% sometimes relied on food parcels or donations 14.8% were short of food or went hungry Another 14.9% sometimes went hungry
Space to study	49.9% had place to study 20.8% sometimes had place to study 29.3% had no place to study
Perception of COVID-19 risk	15.3% felt they were at high risk of COVID-19 These tended to be young people with co-morbidities 31.4% rated their risk as moderate More than half (53.3%) felt they were at low risk These were confident they were taking the right precautions
Biggest challenges experienced	Loss of study time: 57.9% Lack of money for personal and study needs: 55.8% Loss of social contact with family and friends: 42.2% Insufficient money for food: 40.1%
Means to access internet	Data bundle provided by institution: 42.7% Purchase of prepaid data bundle: 29.2% ADSL or fibre at home: 14.3%
Access to virtual learning	Private colleges: 90.2% Universities: 80.2% SETA/other institutions: 72.2% Universities of technology: 62.2% TVET colleges: 38.1% Two-thirds of those with access rated virtual learning portals as good or excellent
Ease of communication with institutions	With institution as a whole: 49.3% gave "easy" rating With lecturers: 66.6% said easy With IT support: 39% found it easy
Confidence in ability to cope with workload after lockdown	Will cope: 41.8% Won't cope: 19.4% Unsure: 38.7%
Sources of social and emotional support	Family: 69.8% Friends: 57.7% Online community: 28.7% A concerning 14.3% said "nobody" as they had been alone
Psychological distress (As measured on 10-item Kessler scale)	More than 65% indicated a degree of distress, ranging from mild to severe Those aged 25+ years were least distressed Women experienced more stress, as did those who considered they were at high risk of COVID-19 infection
Exposure to violence	21.2% experienced some form of physical violence Of these, one in four said violence had been more frequent during lockdown



3.3 DELIVERY OF CORE PROGRAMMES

In both the university and TVET college sub-sectors, health programmes associated with Higher Health had sufficiently strong roots to survive the interruption of contact teaching and learning and the restrictive conditions that applied once physical classes resumed. Adaptability was the key to their continued delivery.

The use of digital meeting platforms and telephonic counselling services, the application of mask wearing, sanitising and social distancing protocols in classroom health and wellness sessions, and the scaling down of First Things First health and wellness events were all key to continuity of the programmes.

Higher Health reports on the outputs of core programmes according to academic year rather than the financial year. This is because our implementation planning follows the academic year.

When the figures for 2020 are reviewed, it should be born in mind that those for the first quarter refer to the pre-COVID era. It so happens, that health activities at universities and TVET colleges got off to a positive start in 2020, with many institutions capitalising on the registration and orientation period to make a strong showing. While the early months of the pandemic in South Africa disrupted Higher Health's intended schedule entirely, in the second half of 2020 the organisation and its campus partners came to terms with the new rules of operation and revived the core programmes that address diverse health issues.

Programme output for 2020 was higher than in 2019 on some indicators and lower on others. This was not unexpected: campus life and Higher Health's partner organisations were affected in diverse ways. It was possible to compensate for some impacts but not for all. However, Higher Health continued to expand its services, the most notable innovations (apart from COVID-related activities) being:

- The launch of a telephonic mental health service and the recruitment of mental health professionals to the staff of Higher Health.
- The commissioning of 10 mobile clinics with staff capable of offering a range of services to students attending campuses where there are no established clinical services.

3.3.1 KNOWLEDGE TRANSFER BEYOND THE OFFICIAL CURRICULUM

Higher Health has always regarded the sharing of knowledge as the first step to enabling individuals to take control of their health and wellbeing. It is the necessary – but insufficient – condition for health empowerment. Over the years, Higher Health has created two major categories of knowledge transfer:

— The Second Curriculum

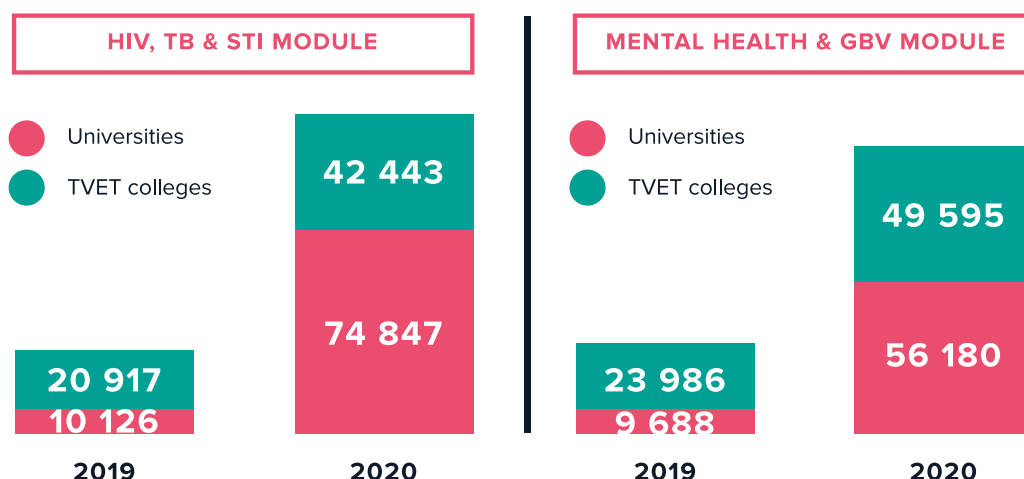
Information is imparted through quite formal methods in classroom conditions – and in the last year through webinars – and is delivered by a trained facilitator. Higher Health has developed two Second Curriculum modules, one dealing with HIV, TB and STIs and the other with mental health and GBV.

— The Extra-Mural Curriculum

This is a collection of less structured learning opportunities, usually employing peer-to-peer methods, and often coupled with highly accessible screening and testing for HIV, TB, and non-communicable diseases (NCDs), and the supply of health products, such as condoms. Peer-driven First Things First health and wellness days are at the heart of the Extra-Mural Curriculum.

In recent years there has been a strategic shift in Higher Health, with more emphasis placed on development of the Second Curriculum. This is reflected in the most recent figures. Despite the COVID-19-related constraints, the combined participation in the HIV/TB/STI and mental health/GBV modules in 2020 was 223 065, more than three times the number in 2019 (64 717). The rate of participation was particularly strong at TVET colleges, where CHWCs play a major role in delivering the Second Curriculum. The use of virtual learning platforms alongside contact sessions made this increased participation possible.

Figure 7: Participation in Second Curriculum modules



The number of student health and wellness days organised under the banner of First Things First declined from 1 779 in 2019 to 1 016 in 2020 – a drop of 43% – largely due to restrictions on gatherings in terms of COVID-19 regulations. The fact that even this number was achieved in 2020 was due to the strong start before the pandemic reached South Africa, and some creative adaptations to the First Things First method after the return to campuses when the first wave of infection passed.

3.3.2 PRACTICAL LEARNING: TRANSLATING KNOWLEDGE INTO ACTION

The Higher Health model encourages students to take a number of health-seeking actions:

- Complete a self-administered assessment for particular health risks.
- Undergo health screening and, where rapid tests are available, testing.
- Seek further healthcare if screening and/or rapid testing so indicates.
- Utilise health products that reduce the risk of infection and unwanted pregnancy.

Bridging the gap between knowledge and action is one of the key challenges in health promotion and these small, achievable steps are designed to start the transition to students taking responsibility for their wellbeing.

SELF-ADMINISTERED RISK ASSESSMENT

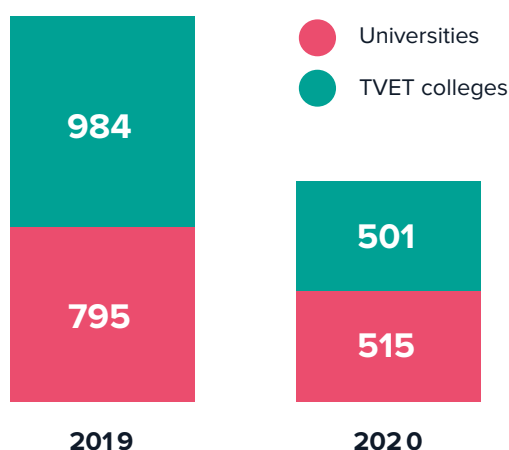
Higher Health has developed two self-administered risk assessment tools that enable students to

understand where they are positioned on a continuum of risk, what factors contribute to risk, and when it is advisable to seek professional assistance. They deal with:

- HIV, STIs, SRH and TB.
- Mental health and GBV.

Wherever possible, risk assessment is offered in an environment where there is also access to information on risk reduction and access to relevant services. This may be during a Second Curriculum session, at a health day or during some other engagement with a peer educator. With the right support, vulnerable individuals can actually avoid illness or trauma, or secure early diagnosis and the right treatment.

Figure 8: Number of health and wellness days

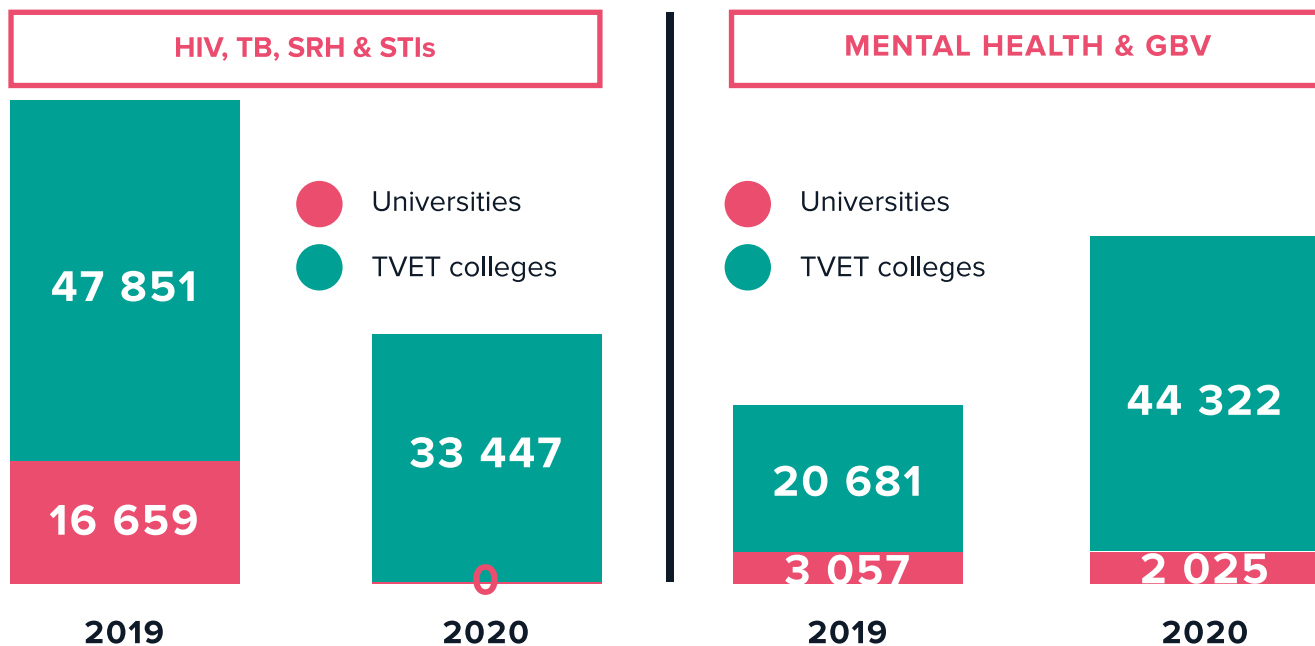




Relative to 2019, there was a decline of about 9% in the total number of self-assessments conducted in 2020 and the total number completed at TVET colleges – 77 769 – far exceeded the number performed by university students (2 025). There was also a clear shift of focus towards the area of mental health and GBV risk. (See Figure 9.)

Presently, self-assessment is a hard-copy exercise, but the COVID-19 HealthCheck success points the way to a more accessible mobile platform for our major health assessments in the future. The only challenge when using an app is to ensure there would still be immediate reassurance and support for those assessed as vulnerable.

Figure 9: Health risk self-assessments completed



SCREENING AND TESTING

Higher Health has promoted and facilitated campus testing and screening for communicable diseases and NCDs for many years. These services are offered in the context of First Things First health and wellness days and throughout the year at campus health facilities. When it comes to the communicable diseases – HIV, TB and STIs – treatment is also a major factor in curtailing the spread of infection and is positioned as such in the National Strategic Plan on HIV, TB and STIs. In South Africa, the “prevention bonus” that flows from identifying and successfully treating the vast majority of people living with HIV has been a major factor in reducing new infections.

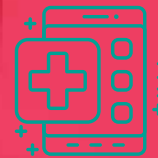
Over a sustained period, student testing and screening numbers climbed steadily. In 2019, there was a modest decline in numbers as Higher Health focused on

building its network of regional and provincial offices, and recruiting and training CHWCs. However, during 2020, COVID-19 dealt the campus screening and testing programme a much heavier blow.

Overall, HIV tests in 2020 were down 48% year-on-year and TB and STI screenings down 50%, while NCD screenings decreased by 29%. NCD screenings include screening for diabetes, high blood pressure and high cholesterol.

The obvious reason for this was that students were not on campus at all for a few months and that the daily student presence on campus was relatively



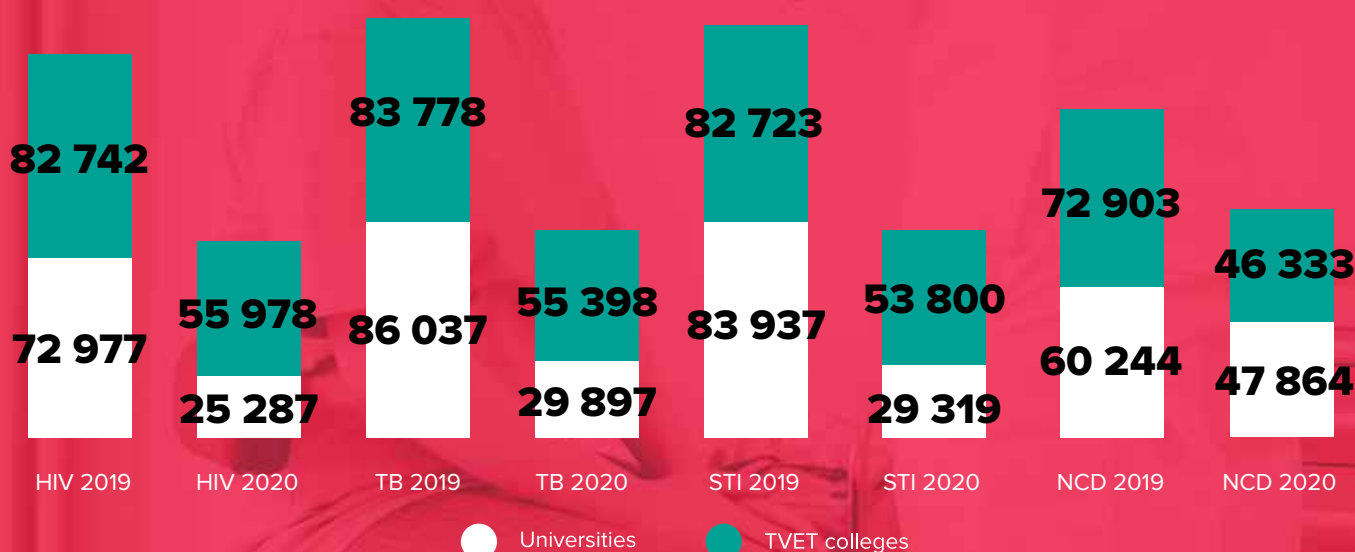


low even after face-to-face classes resumed. The fact that the decline in screening and testing for HIV, TB and STIs affected TVET colleges less severely tends to support this, as a higher proportion of students in this sub-sector returned to classes virtually full-time. The TVET sector also recruited and trained a higher number of COVID-19 screening volunteers who later incorporated other screening into their activities.

However, it should also be noted that a decline in utilisation of non-COVID, non-emergency health

services was noted across the country in all demographic groups during the year and this was apparently due to a public perception that visiting a health facility constituted a potential COVID risk. National and international public health experts have expressed concern about the adverse impact of this on national HIV and TB programmes, and Higher Health is anxious to correct the downturn in screening and testing in the coming year – despite the continued presence of the COVID-19 pandemic.

Figure 10: Number of tests and screenings performed on campus



ACCESSING PSYCHOSOCIAL SUPPORT

Higher Health formed a partnership with the South African Depression and Anxiety Group (SADAG) to offer a 24-hour toll-free helpline providing psychosocial support to students often at moments of personal crisis. The line was launched towards the end of July 2020 and is part of a broader mental health programme outlined on [page 36](#).

Between August and December 2020, the number of calls totalled 4 277, averaging 855 a month. A further 354 individuals accessed the service by SMS. The pattern in the first quarter of 2021 has been similar, with a total of 2 801 calls and 260 SMS users.

It has been encouraging to see that men who are usually reluctant healthcare consumers comprise the majority of helpline users (69%). Higher Health particularly aimed to reach male students as men in South Africa are four times more likely to commit suicide than women and this speaks to a failure of mental healthcare.

Helpline users are provided with initial counselling and in many cases referred to various services for continued care. The largest number – 30% of callers – were referred for crisis counselling, 19% to student support services, and 16% to government health facilities. Psychologists on the staff of institutions and Higher Health received many of these referrals.



Figure 11: Use of helpline for psychosocial support August to December 2020

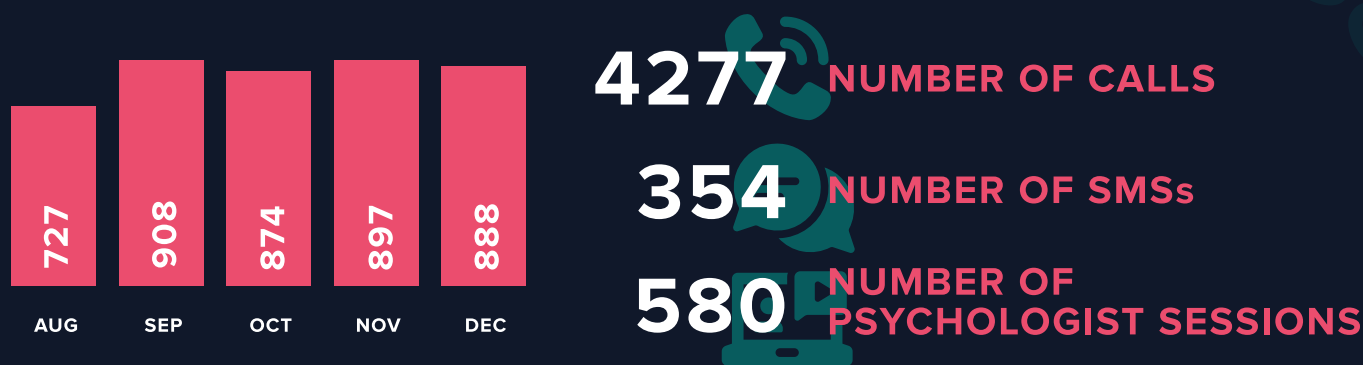


Figure 12: Reasons for using helpline



3.3.3 ACCESS TO SERVICES

Higher Health has contributed substantially to the development of accessible and appropriate services for students:

- It has facilitated partnerships with provincial and district health services to establish clinics on campuses where there have been none. The response of the public health system has been extremely positive and we have seen new fixed clinics built as well as personnel supplied to run part-time clinics in available college structures.
- It has enabled established campus health services to reposition their services to meet a range of additional health needs of young people. In particular, Higher Health has provided health worker training on antiretroviral treatment (ART), reproductive health and mental health. Partnerships with public health authorities have resulted in clinics at universities and TVET colleges receiving regular stocks of HIV test kits, ARVs, contraceptives and other medical consumables.

These arrangements have proved sustainable and have given hundreds of thousands of students the right services in the right place at the right time. Yet, gaps persisted particularly at TVET college campuses in more remote areas.

MOBILE CLINIC FLEET

In 2020, Higher Health – with the assistance of the Department of Science and Technology – leased 10 fully equipped mobile clinics to service these TVET colleges on a roster basis and to begin incorporating CET colleges into the service net. The frequency of mobile clinic visits depends on the size of the campus, with main campuses holding weekly clinics and satellite campuses receiving a fortnightly or monthly clinic visit.

Services are closely aligned with national public sector clinical guidelines and reporting systems.

An important aspect of this service has been the incorporation of psychosocial services into basic clinic care. Each mobile team comprises a nurse, a psychologist and two community health workers, all of whom are Higher Health staff members.



ESSENTIAL CLINIC SERVICES

Screening and testing
Prevention of HIV, TB, STIs and GBV
Linkage to laboratory testing
Dispensing of chronic medicine
Referral to public health facilities
Referral for psychosocial support

PSYCHOSOCIAL SUPPORT PROGRAMME

Reference has already been made to the toll-free helpline, the incorporation of clinical psychologists into mobile clinic teams, and the prioritisation of mental health within the Second Curriculum and students' self-assessment of their health risks. These are important components of Higher Health's model of psychosocial support which is gradually being implemented across the sector. (See Figure 13.)

The success of the Higher Health model depends on how well interventions at the various levels feed into each other. It is vital that students who are identified as vulnerable through self-assessment and helpline callers who require more than crisis counselling are able to access sustained professional care.

Higher Health employs a total of six clinical psychologists to reach out to under-served campuses. A number of institutions provide access to mental health professionals through their student counselling services and campus health facilities. However, there is still a considerable way to go in developing the mental health programme.

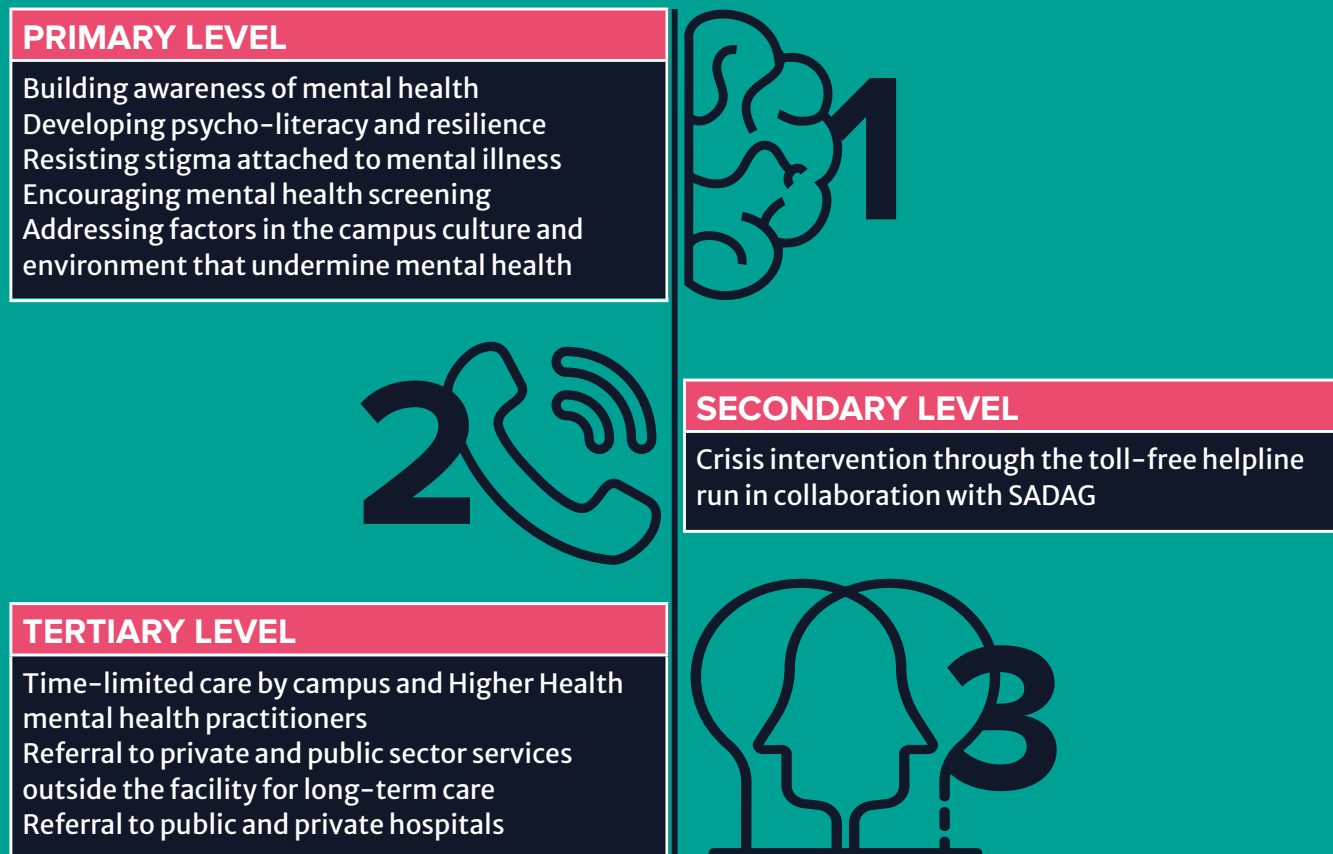
Table 5: Service areas of mobile clinics

AREA	NUMBER OF MOBILE TEAMS
Free State	1
Eastern Cape	2
Gauteng and North West	2
KwaZulu-Natal	2
Limpopo	1
Mpumalanga	1
Northern and Western Cape	1





Figure 13: Higher Health's tiered model for psychosocial support



SEXUAL AND REPRODUCTIVE HEALTH

Given the average age of the student population, sexual and reproductive health (SRH) is an important aspect of campus clinic services. Higher Health aims to facilitate a service that offers young people choices and guarantees confidentiality. The service offering includes counselling on and supply of contraception, pregnancy testing and referral for antenatal care or termination of pregnancy,

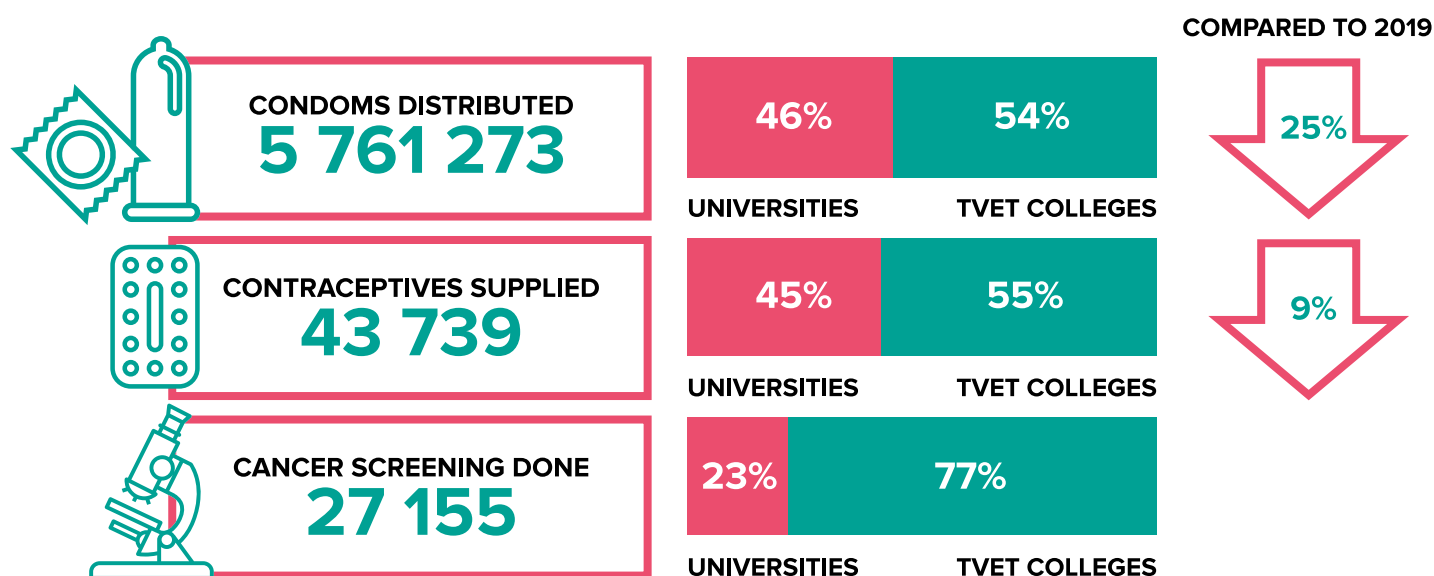
prevention and treatment of STIs, and screening for breast, cervical and prostate cancer.

We are mindful that many young women in South Africa bear their first child in their late teens and that these pregnancies are frequently unplanned. Higher Health's goal is to ensure that no college or university student has to abandon her studies because of an unplanned pregnancy.

There was a year-on-year decline in the quantity of SRH products supplied in 2020, almost certainly related to COVID-19 campus closures.



Figure 14: Selected SRH products and services supplied in 2020



3.3.4 FOCUS ON VULNERABLE GROUPS

Within the broad student population, Higher Health has programmes that focus on particular groups because of their heightened vulnerability to identified health risks, their susceptibility to stigma and discrimination, and their potential to be excluded from essential health services. These groups are:

- Young women.
- LGBTQI+ students.
- Students with disabilities.

All our programmes address human rights issues relating to students from these groups and educate healthcare workers about their particular health needs. Our focus on GBV is of benefit both to young women and LGBTQI+ students, whose gender and sexual orientation may expose them to verbal and physical abuse.

GLOBAL FUND PROGRAMME FOR ADOLESCENT GIRLS AND YOUNG WOMEN (AGYW)

Higher Health has received a three-year funding grant from the Global Fund to Fight AIDS, TB and Malaria for a programme focused on young women under the age of 25 years attending TVET colleges. The period April 2020 to March 2021 represented year two of the grant.

The programme aims to reduce young women's vulnerability to HIV, protect them from unwanted early pregnancy, lower their risk of GBV, and increase their chances of completing their chosen course of study. The initiative combines interventions to address behavioural and structural factors impacting on health with improving access to a defined package of health services.

Participation is open to woman students attending 13 TVET colleges. Higher Health

mostly selected colleges in rural areas, where poverty is generally deeper and community health services less developed. The programme therefore has the potential to make a bigger difference in these settings.

During registration and orientation weeks at the start of each academic year a concerted effort is made at participating TVET colleges to enrol female students in this programme, although participants may join at any stage during the year. Once enrolled, participants are strongly encouraged to participate in all the health activities the college affords them – attendance at Second Curriculum sessions and extramural health days, uptake of testing and screening services, utilisation of the risk

assessment tool, and use of clinic and related services, as required.

In the course of 2021, young women participating in this programme will also have no-cost access to a range of health and feminine hygiene products, including male and female condoms, lubricant, pregnancy tests and sanitary towels. These will be provided by means of vending machines on TVET college campuses which can only be activated by use of a PIN code issued to programme participants.

The programme indicators, specified by the Global Fund, relate mainly to HIV prevention and treatment. (See Table 6.)

Table 6: Global Fund AGYW Programme performance April 2020 to March 2021

	TARGET FOR YEAR 2	ACHIEVED IN YEAR 2
Number of young women reached by defined package of HIV prevention services*	19 970	16 376
Number of young women who tested for HIV and received results during the reporting period	15 976	11 518
Number of young women newly diagnosed with HIV who were linked to HIV care	717	28

*HIV and GBV risk screening, offer of HIV test and condoms, provision of information on HIV, SRH and GBV, individual plans for linkage to care

PROGRAMME ON GENDER-BASED VIOLENCE AND FEMICIDE

At various points in this report on Higher Health's performance, there has been reference to addressing GBV. It forms part of the Second Curriculum, features in self-administered risk-assessment tools, and the toll-free helpline is geared to deal with crises related to GBV. Important as these interventions are, Higher Health recognises the need to tackle this malignant feature of our society in a more integrated manner.

Developments during 2020/21 have set the scene for this:

- In July 2020, the DHET published the Policy Framework to Address Gender-Based Violence in the Post-School Education and Training Sector. Higher Health had been instrumental in the development of the framework.
- On 8 August 2020, the framework was launched at a digital event chaired jointly by the Deputy Minister of Higher Education, Mr Buti Manamela, and the Deputy Minister of Women, Youth and Persons with Disabilities, Prof Hlengiwe Mkhize.
- In November 2020, a model for the sector's GBV programme was approved by the Higher Health Board which also gave the green light for the establishment of a GBV Technical Task Team to guide implementation of the programme and monitor its progress. Membership of the task team was being finalised at the end of this reporting period.
- By the end of March 2021, the first drafts of the following policy documents had been produced:
 - A code of ethics for staff and students at institutions.
 - A policy framework on sexual and gender misconduct.
 - Protocols on safety, response to rape and security measures.

The country's response to the COVID-19 pandemic provided some useful lessons for the GBV Programme. It underscored the indispensable role of political commitment and leadership at all levels – and how a sense of crisis can play a powerful mobilising role.

GBV is a social disease with roots that go deep and spread wide. It requires much more than a multi-disciplinary programme – it demands the activation of entire campuses and the commitment of various centres of power in institutions of learning. In 2021/22, Higher Health will be casting its net more widely within the sector to bring crucial role players on board and harness their skills and influence.

3.3.5 UNIVERSITY GRANTS PROGRAMME

While Higher Health provides direct assistance to TVET colleges through the placement of CHWCs on campuses and the provision of mobile clinics, it supports universities less directly through the allocation of a monetary grant that is to be used to implement the Higher Health Model. All universities also invest some of their own resources in student health and wellness, so the Higher Health grant is intended to supplement the universities' own investment.

Institutions are invited to apply for the Higher Health grant, and amounts are determined partly by the activities and targets indicated in the application as well as by the size of the institution and equity considerations. The latest grant allocation was made for a two-year period, from April 2019 to March 2021, and individual grants ranged from R250 000 to R500 000.

The goal was for all 26 universities to utilise the entire grant by the end of this period, but the COVID-19 effect was also apparent in this area. Although three universities had spent 90% or more of the amount by 31 March 2021, the average was a mere 56%.

In April 2020, Higher Health gave universities the option of reallocating their grants to COVID-19 interventions and a majority elected to do so. The results were uneven: some institutions achieved

extensive training of staff and students to undertake COVID-19 screening and promote compliance with safety protocols, while output at other institutions was disappointing.

Higher Health also monitors core programme output, and it was apparent that performance had been adversely affected at most institutions.

In addition to applying for new two-year allocations in the next financial year, universities will be permitted to carry their unspent funding for 2019–2021 into the next year, provided they submit a revised plan for utilisation of the amount carried forward.



Table 7: Percent of grant performance targets met by universities

60% - 100%	40% - 59%	0% - 39%
9 UNIVERSITIES	7 UNIVERSITIES	9 UNIVERSITIES

Note: one university was excluded from this analysis because its grant was withheld pending report submission

3.3.6 COMMUNICATION PROGRAMME

The Communication Programme switched into high gear to respond to the need for accurate and credible information in this period of crisis. Higher Health was well aware that disinformation will flourish, if given a chance, when societies are thrown into turmoil. While national government's communication as the pandemic reached South Africa was clear and timely, students and staff in the post-school education and training sector also needed more specific information related to their circumstances. Higher Health worked alongside the Department of Higher Education and Training, Minister Nzimande and Deputy Minister Manamela to provide information and guidance.

Messaging and channels of communication evolved along with the pattern of disease and changing regulations promulgated under the national state of disaster. Overall, Higher Health communications made a strong shift to digital and mobile technology and intensified engagement with the mass media.

Quick off the mark: social media and SMS campaign
Higher Health had the means to deliver a sustained series of SMS messages during the first three months of the pandemic, when students were away from campus and attempting to study from home. Some

6 588 142 individual texts were sent between April and June 2020, with each message reaching about 680 000 students. Special attention was paid to the quality of content and more complex messaging was made accessible by linking to short video clips.

The Higher Health Facebook page was redeveloped in April 2020 and ready to be deployed in the COVID response. Higher Health already had a healthy Twitter following and, with daily postings, managed to boost this to more than 600 000 by October 2020.

Making common cause with journalists

South African mass media played a significant and constructive role in informing and reassuring the public as the pandemic spread to our country. Higher Health's traditional media relations activities, involving television, radio and online news channels, yielded extremely positive results in terms of getting important messages to students. The organisation's CEO participated in several media briefings and events alongside the Minister Nzimande.



The total number of readers, viewers and listeners of media carrying reports on Higher Health amounted to 118 973 616. (Many digital newspapers, radio stations and TV channels covered Higher Health on multiple occasions and this accounts for the cumulative media reach exceeding the total population.) COVID-19 was a sustained story and, for once, the media were keen to educate as well as feed the public appetite for news.

Tapping the power of digital events

Higher Health staged a number of virtual events or webinars at intervals throughout the year. Some were

directly COVID-related while others continued long-standing Higher Health discussions – for example, on the prevention of GBV and the need to prioritise mental health. The organisation managed to attract outstanding presenters and panel members, and used social media successfully to market the events.

Higher Health CEO Prof Ahluwalia also participated as a speaker in several webinars hosted by other organisations and presented in webinars that targeted more specialised audiences.

Table 8: Major webinars hosted by Higher Health

DATE	TOPIC	PARTICIPATION
21 April	Impact of COVID-19 on youth	5 000+
7 July	Guidelines and protocols for COVID-19 in post-school education and training sector	500+
8 August	GBV in the post-school education and training sector	1 000+
21 October	Mental health the key to graduate success	2 500+
27 January*	Managing the second COVID-19 wave in the post-school education and training sector	1 855+
13 March	Lifeskills empowerment of tertiary level students	1 000+

*This was hosted jointly with Minister of Higher Education, Science and Technology and a range of other organisations using multiple digital links. The figure for participation related only to the Higher Health link.

MATERIALS DEVELOPMENT

During 2020, Higher Health procured and distributed more than 25 000 posters and a million leaflets to universities and colleges. These explained the standard, simple prevention measures to both students and staff. Placed strategically in areas of high foot-traffic – such

as entrances, residences, cafeterias, stair wells – posters served to keep the need for constant vigilance top of mind.

Visits to campuses confirmed that they were widely used and clearly filled a need. A second mass delivery of posters and leaflets was made in early 2021.

Table 9: Information, education and communication materials distributed

	UNIVERSITIES		TVET COLLEGES		COMMUNITY COLLEGES	
	2020	2021	2020	2021	2020	2021
Posters	12 710	10 000	10 140	10 000	2 300	2 000
Pamphlets	521 900	500 000	465 400	400 000	109 000	100 000



FUTURE BEATS KEEPS ON GIVING

One of Higher Health's enduring initiatives is Future Beats, a youth-driven radio and social media intervention. The organisation provides a modest amount of funding and offers skills training to producers and presenters of campus and community radio stations to enable them to incorporate health and related social content in their programming. Over the years this has resulted in creative and engaging communication.

In 2020, there were 21 radio stations in the Future Beats network, with a total listenership estimated at slightly over a million and nearly 300 000 social media followers. Higher Health ran special webinars for more than 200 journalists at these stations to ensure they had a good grasp of the facts of COVID-19 transmission and prevention, as well as when and how to access

testing and treatment. The journalists played an invaluable role not only in sharing information but also motivating students to take appropriate action.

More broadly, Future Beats stations promote health and wellbeing throughout the year and follow a common set of monthly themes and weekly topics. These relate partly to international and national health days and are also geared to the rhythms of student life – for example, the admission of new students and the stresses of exam time. Podcasts of health and related social programmes are available on social media of the stations.

Community stations with a strong youth listenership comprise one-third of the total number comprising Future Beats. They are mostly situated in districts where there are tertiary education and training institutions but no campus radio stations.

Table 10: Radio stations comprising the Future Beats initiative 2020

STATION	CATEGORY	LOCATION	LISTENERSHIP
VOW FM	Campus	Johannesburg, GP	20 000
Maputaland Radio	Community	Northern KZN	85 000
Tshwane FM	Campus	Tshwane, GP	38 000
Durban Youth Radio	Community	eThekweni, KZN	12 000
Unitra FM	Campus	OR Tambo District, EC	240 000
Madibaz FM	Campus	Nelson Mandela Bay, EC	15 000
Forte FM	Campus	OR Tambo District, EC	7 000
Radio DUT	Campus	eThekweni, KZN	8 000
UNISA Radio	Campus	Pretoria, GP	15 000
Rhodes Music Radio	Campus	Eastern Cape	21 838
Alfred Nzo	Community	Eastern Cape	138 000
CUT FM	Campus	Bloemfontein, FS	38 000
Kovsie FM	Campus	Bloemfontein, FS	28 003
Emalahleni FM	Community	Witbank, MP	97 000
Barberton Radio	Community	Mpumalanga	55 000
Univen Radio	Campus	Venda, LP	9 000
Radio Turf	Campus	Polokwane, LP	6 000
UCT Radio	Campus	Cape Town, WC	16 899
Mdantsane FM	Community	Buffalo City, EC	3 900
Mahikeng FM	Community	Mahikeng, NW	88 000
Zululand Radio	Campus	Zululand, KZN	38 000
TOTAL			1 020 841

OVERSIGHT AND VISIBLE SUPPORT TO INSTITUTIONS

The political heads and officials of the DHET visited a substantial number of institutions during the course of the reporting year to gain assurance that protocols were being implemented and to demonstrate support for institutional leadership.

Deputy Minister Manamela, head of the sector's Ministerial COVID-19 Task Team, visited at least 23 campuses while Minister Nzimande undertook further visits. Higher Health's Chief Executive Officer or another member of the executive team was always present during these visits and ready to assist with any follow-up action that might be required.

HUMAN RESOURCE MANAGEMENT



The staff complement of Higher Health continued to grow during 2020/21, although at a slower rate than the previous year. The notable additions were more than 20 members of clinical teams responsible for mobile clinic services in various provinces. These health workers are attached to provincial offices.

Higher Health strives to promote employment equity and the extent to which a representative work force has been achieved is reflected in Figures 16 and 17.

A major human resource concern during the reporting year was keeping Higher Health staff members safe from infection. A relatively small number could work effectively from home. Many were out there, in our regional and provincial offices or on campus, interacting with institutional management, training staff and student volunteers, ensuring COVID-19 protocols were well understood and being implemented, and delivering Second Curriculum sessions.

Management recognised the extent to which our employees were putting themselves at risk for the good of the sector and took practical steps to reduce their risk. These included:

- Ensuring availability of relevant personal protective equipment to staff undertaking diverse responsibilities.
- Adapting national, regional and provincial offices for safe operation.
- Providing training to staff on COVID-19 developments.
- Facilitating access to counselling and funding COVID-19 testing for staff members.

Due to the fact that our funding commitments are short-term, our staff are on fixed-term contracts with life insurance and medical aid benefits.

The salary increment for the year was based on the consumer price index, while bonuses were performance-linked. In addition, staff who were in the frontline of the COVID-19 response were also acknowledged and rewarded.

Figure 15: Employees by occupational category as at 31 March 2021

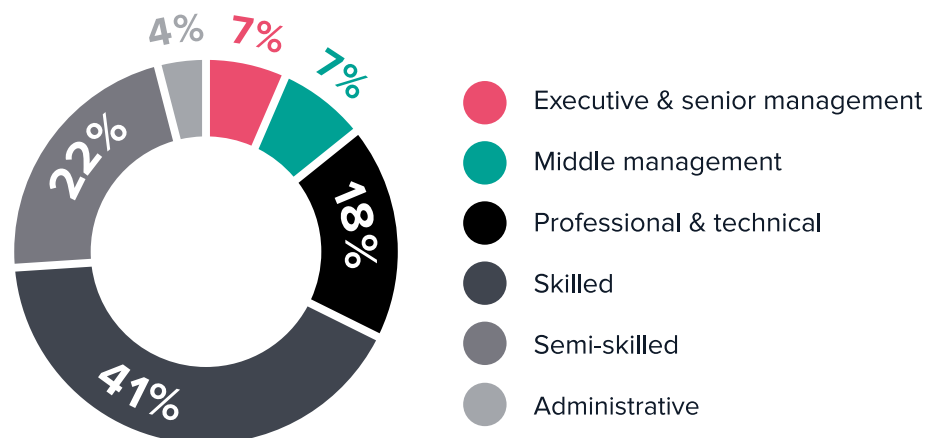


Figure 16: Employees by racial category as at 31 March 2021

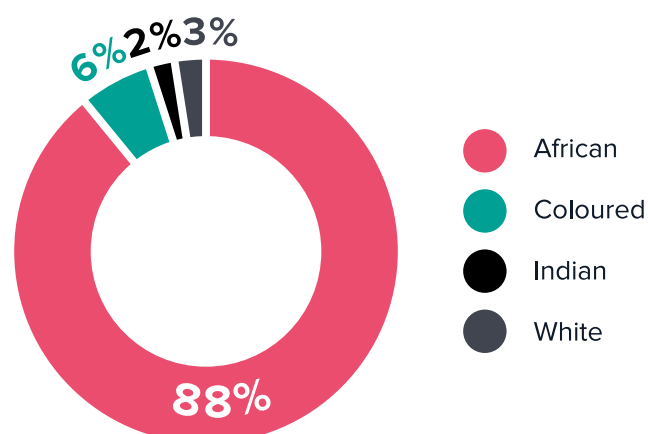
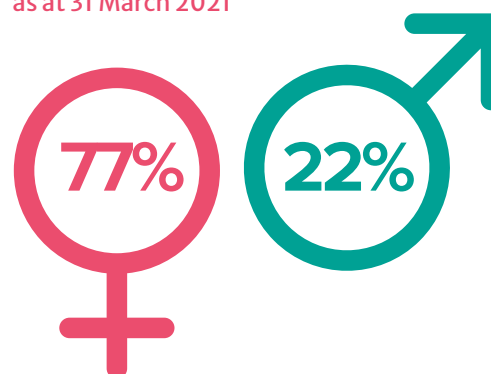


Figure 17: Employees by gender as at 31 March 2021





TOT GOVERNANCE II



Higher Health is a non-profit company, registered in terms of the Companies Act. The strategic direction of the organisation and oversight of its management and performance vest in a Board of Directors.

The Board has been constituted to include representation of stakeholders that have been key to Higher Health's development (USAf, South African Public Colleges Organisation (SAPCO) and the DHET) and to ensure that the organisation has access to individuals with special expertise. The current chairperson is Prof Wim de Villiers, Vice-Chancellor of Stellenbosch University, and the Deputy Chairperson is Mr Sanele Mlotshwa, Rector of Majuba TVET College. A full list of members appears below.

The Board's guidance was especially valuable in the unpredictable environment created by the COVID-19 pandemic. The situation demanded adaptability and some quick decisions, but it was important not to erode good governance in the process. Fortunately, Board members were accessible to provide timeous advice.

In terms of Higher Health's memorandum of incorporation, the Board is required to meet at least three times a year. During 2020/21, it met three times, and on all these occasions used digital meeting platforms.

Table 11: Members of Higher Health Board of Directors

BOARD MEMBER	ORGANISATION REPRESENTED/ AREA OF EXPERTISE
Prof Wim de Villiers – Chairperson Vice-Chancellor, Stellenbosch University	Medical science Management
Mr Sanele Mlotshwa – Deputy Chairperson President of SAPCO and Rector, Majuba TVET College	Representing 50 TVET colleges
Prof Ahmed Cassim Bawa CEO, USAf	Representing 26 universities
Mr Mahlubi Mabizela Chief Director: University Policy & Development Support, DHET	DHET University Branch
Mr Brian Madalane Principal, Northern Cape TVET College	Higher education and training Institutional management
Prof Thokozile Valerie Mayekiso Vice-Chancellor, University of Mpumalanga	Higher education and training Institutional management
Prof Nancy Refilwe Phaswana-Mafuya Professor of Epidemiology and Public Health, University of Johannesburg	Public health Social determinants of health
Mrs Ashwanthee Singh Acting Deputy Director-General, TVET Branch, DHET	DHET TVET Branch
Dr Doeke Tromp Former CFO, Tshwane University of Technology	Financial management Strategic planning
Ex officio	
Dr (Prof) Ramneek Ahluwalia CEO of Higher Health	Higher Health

The Board has two specialised committees which met as necessary during the year.

Table 12: Committees of Higher Health Board

COMMITTEE	CHAIRPERSON	NUMBER OF MEETINGS
Audit & Risk and Finance & Investment Committee	Mr Theo Madurai – Independent CFO, Tshwane University of Technology	Two
Human Resources & Remuneration Committee	Prof Ahmed Bawa USAf representative on Board	Two

FOR THE
YEAR ENDED
31 MARCH
2021

ANNUAL FINANCIAL REPORT



GENERAL INFORMATION

Country of incorporation and domicile	South Africa
Registration number	2017/419731/08
Nature of business and principal activities	Health and wellness services and capacity development in higher education institutions, which include universities and TVET colleges
Directors	Prof Wim De Villiers Dr (Prof) Ramneek Ahluwalia Prof Nancy Metse Phaswana-Mafuya Prof Thokozile Valerie Mayekiso Dr Doeke Tromp Mr Mahlubi Mabizela Mrs Ashwanthee Singh Prof Ahmed Bawa Mr Brian Madalane Mr Sanele Mlotshwa
Registered office	262 Rose Avenue Wild Olive, Riverfalls Office Park Centurion 0132
Business address	262 Rose Avenue Wild Olive, Riverfalls Office Park Centurion 0157
Postal address	262 Rose Avenue Wild Olive, Riverfalls Office Park Centurion Pretoria 0132
Bank	ABSA Bank Limited
Level of assurance	These financial statements have been audited in compliance with the applicable requirements of the Companies Act 71 of 2008 South Africa.
Auditor	PricewaterhouseCoopers Inc Chartered accountants (SA) Registered auditors
Preparer	The annual financial statements were internally compiled by George Chetty



DIRECTORS' RESPONSIBILITIES AND APPROVAL

The directors are required by the Companies Act 71 of 2008 South Africa to maintain adequate accounting records and are responsible for the content and integrity of the annual financial statements and related financial information included in this report. It is their responsibility to ensure that the annual financial statements satisfy financial reporting standards with regard to form and content and present fairly the statement of financial position, results of operations and business of the company, and explain the transactions and financial position of the business of the company at the end of the financial year. The annual financial report is based on appropriate accounting policies, consistently applied throughout the company and supported by reasonable and prudent judgements and estimates.

The directors acknowledge that they are ultimately responsible for the system of internal financial control established by the company and place considerable importance on maintaining a strong control environment. To enable the directors to meet these responsibilities, the Board of Directors sets standards for internal control aimed at reducing the risk of error or loss in a cost-effective manner. Standards include the proper delegation of responsibilities within a clearly defined framework, effective accounting procedures and adequate segregation of duties to ensure an acceptable level of risk. These controls are monitored throughout the company and all employees are required to maintain the highest ethical standards in ensuring the company's business is conducted in a manner that, in all reasonable circumstances, is above reproach.

The focus of risk management in the company is on identifying, assessing, managing and monitoring all known forms of risk across the company. While operating risk cannot be fully eliminated, the company endeavours to minimise it by ensuring that appropriate infrastructure, controls, systems and ethical behaviour are applied and managed within predetermined procedures and constraints.

The directors are of the opinion, based on the information and explanations given by management, that the system of internal control provides reasonable assurance that the financial records may be relied on for the preparation of the annual financial statements. However, any system of internal financial control can provide only reasonable, and not absolute, assurance against material misstatement or loss. The going-concern basis has been adopted in preparing the financial statements. Based on forecasts and available cash resources the directors have no reason to believe that the company will not be a going concern in the foreseeable future. The financial report supports the viability of the company.

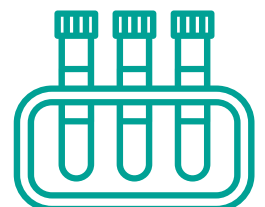
The annual financial report set out on pages **58** to **60**, and the supplementary information set out on page **52** to **54**, which have been prepared on the going concern basis, were approved by the Board of Directors and were signed on 6 September 2021 on their behalf by:



CHAIRPERSON



CHIEF EXECUTIVE OFFICER



REPORT OF THE AUDIT & RISK AND FINANCE & INVESTMENT COMMITTEE

We are pleased to present our report for the financial year ended 31 March 2021.

The audit risk finance and investment committee is an independent statutory committee appointed by the directors of the company. The committee has conducted its affairs in compliance with its terms of reference.

1. AUDIT RISK FINANCE AND INVESTMENT COMMITTEE MEMBERS

The audit & risk and finance & investment committee is independent and consists of five independent, non-executive directors. It meets at least twice a year as per its terms of reference.

The chairman of the Board, chief executive officer, finance director, external auditor and other assurance providers (legal, compliance, risk, health and safety) attend meetings by invitation only. Members of the committee are:

Name of member	Position	Appointed	Institution
Mr Theo Madurai	Chairperson	21/11/2017	Tshwane University of Technology – Chief Financial Officer
Prof Thokozile Mayekiso		21/11/2017	University of Mpumalanga – Vice Chancellor
Dr Doeke Tromp		21/11/2017	Independent – ex CFO of Tshwane University of Technology
Mr Eminos Manyawi		15/03/2019	Sefako Makgatho Health Sciences University – Chief Financial Officer
Mr Brian Madalane		15/03/2019	Northern Cape TVET College – Principal

2. MEETINGS HELD BY THE AUDIT & RISK AND FINANCE & INVESTMENT COMMITTEE

The Audit & Risk and Finance & Investment Committee performs the duties laid upon it by section 94(7) of the Companies Act 71 of 2008 by holding meetings with key role players on a regular basis and by the unrestricted access granted to external auditor.

as set out in section 94(8) of the Companies Act 71 of 2008 South Africa, which includes consideration of previous appointments of the auditor, the extent of other work undertaken by the auditor for the company and compliance with criteria relating to independence or conflicts of interest as prescribed by the Independent Regulatory Board for Auditors. Requisite assurance was sought and provided by the auditor that internal governance processes within the audit firm support and demonstrate its claim to independence.

3. EXTERNAL AUDITOR

The audit committee has satisfied itself that the external auditor was independent of the company,

The committee ensured that the appointment of the auditor complied with the Companies Act 71 of 2008 South Africa, and any other legislation relating to the appointment of auditors.

The committee, in consultation with executive management, agreed to the engagement letter, terms, audit plan and budgeted audit fees for the 2021 year.

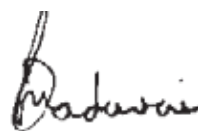
4. ANNUAL FINANCIAL STATEMENTS

The committee has reviewed the accounting policies and the financial statements of the company and is satisfied that they are appropriate and comply with International Financial Reporting Standards.

5. EFFECTIVENESS OF INTERNAL CONTROL

The system of internal control applied by the entity over financial reporting and risk management is effective, efficient and transparent. From the audit report on the financial statements and the

management report of the external auditor it was noted that no matters were reported that indicate any material deficiencies in the system of internal control or any deviations therefrom. Accordingly, the committee can report that the system of internal control over financial reporting for the period under review was efficient and effective.



THEO MADURAI
**CHAIRPERSON OF THE AUDIT
& RISK AND FINANCE &
INVESTMENT COMMITTEE**

6 September 2021

DIRECTORS' REPORT

The directors present their report for the year ended 31 March 2021.

1. REVIEW OF FINANCIAL RESULTS AND ACTIVITIES

MAIN BUSINESS AND OPERATIONS

The company facilitated health and wellness services and capacity development in higher education institutions which included universities and TVET colleges. There were no major changes herein during the year.

The operating results and statement of financial position of the company are fully set out in the attached financial statements and do not in our opinion require any further comment.

2. GOING CONCERN

The annual financial statements have been prepared on the basis of accounting policies applicable to a going concern. This basis presumes that funds will be

available to finance future operations and that the realisation of assets and settlement of liabilities, contingent obligations and commitments will occur in the ordinary course of business.

The directors believe that the company has adequate financial resources to continue in operation for the foreseeable future and accordingly the annual financial statements have been prepared on a going concern basis.

The directors have given due consideration to the potential impact of the COVID-19 pandemic on the company's ability to continue as a going concern. The directors believe that the pandemic will have a temporary impact on the business activities. Notwithstanding these short-term challenges, the directors are of the view that the company has sufficient resources to continue as a going concern.

The ability of the company to continue as a going concern is dependent on a number of factors.

The most significant of these is that the directors continue to procure funding for the ongoing operations for the company where the need arises. Management is comfortable that Higher Health, will continue to operate.

Further, Higher Health is actively involved in fighting the pandemic and has been granted substantial funding to continue do so. Higher Health is in the forefront of rolling out a vaccination strategy especially in the post-school education sector.

The company incurred a net surplus for the year ended 31 March 2021 of R3 795 923 (2020 surplus: R2 915 834). The company continues to generate surpluses.

3. EVENTS AFTER REPORTING DATE

All events subsequent to the date of the annual financial statements and for which the applicable financial reporting framework requires adjustment or disclosure have been adjusted or disclosed.

The directors are not aware of any matter or circumstance arising since the end of the financial year to the date of this report that could have a material effect on the financial position of the company.

The company has assessed the impact of COVID-19 on the annual financial statements and considered the potential impact on the business. While it is envisaged that there may be a negative impact on the performance of the company over the remainder of the financial year, the full impact of COVID-19 cannot be reasonably estimated at this time. The full impact of the COVID-19 outbreak continues to evolve at the date of this report.

4. DIRECTORS' INTEREST IN CONTRACTS

To our knowledge none of the directors had any interest in contracts entered into during the year under review.

5. DIRECTORS

The directors of the company during the year and up to the date of this report are as follows:

Prof Wim De Villiers
Dr (Prof) Ramneek Ahluwalia
Prof Nancy Metse Phaswana-Mafuya
Prof Thokozile Valerie Mayekiso
Dr Doeke Tromp
Mr Mahlubi Mabizela
Mrs Ashwanthee Singh
Prof Ahmed Bawa
Mr Brian Madalane
Mr Sanele Mlotshwa

6. SECRETARY

No secretary has been formally appointed during the current financial year.

7. PROPERTY, PLANT AND EQUIPMENT

There were no changes in the nature of property, plant and equipment of the company or policy regarding their use.

8. INDEPENDENT AUDITORS

PricewaterhouseCoopers Inc were the independent auditors for the year under review.





Independent auditor's report

To the Members of Higher Education and Training Health, Wellness and Development Centre NPC

Our opinion

In our opinion, the financial statements present fairly, in all material respects, the financial position of Higher Education and Training Health, Wellness and Development Centre NPC (the Company) as at 31 March 2021, and its financial performance and cash flows for the year then ended in accordance with the International Financial Reporting Standard for Small and Medium-sized Entities and the requirements of the Companies Act of South Africa.

What we have audited

Higher Education Training Health Wellness Development Centre NPC's financial statements set out on pages 10 to 30 comprise:

- the statement of financial position as at 31 March 2021;
- the statement of comprehensive income for the year then ended;
- the statement of changes in equity for the year then ended;
- the statement of cash flows for the year then ended;
- accounting policies; and
- the notes to the financial statements.

Basis for opinion

We conducted our audit in accordance with International Standards on Auditing (ISAs). Our responsibilities under those standards are further described in the *Auditor's responsibilities for the audit of the financial statements* section of our report.

We believe that the audit evidence we have obtained is sufficient and appropriate to provide a basis for our opinion.

Independence

We are independent of the Company in accordance with the Independent Regulatory Board for Auditors' *Code of Professional Conduct for Registered Auditors* (IRBA Code) and other independence requirements applicable to performing audits of financial statements in South Africa. We have fulfilled our other ethical responsibilities in accordance with the IRBA Code and in accordance with other ethical requirements applicable to performing audits in South Africa. The IRBA Code is consistent with the corresponding sections of the International Ethics Standards Board for Accountants' *International Code of Ethics for Professional Accountants (including International Independence Standards)*.

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Chief Executive Officer: L S Machaba
The Company's principal place of business is at 4 Lisbon Lane, Waterfall City, Jukskei View, where a list of directors' names is available for inspection.
Reg. no. 1998/012055/21, VAT reg.no. 4960174682.

Other information

The directors are responsible for the other information. The other information comprises the information included in the document titled “Higher Education Training Health Wellness Development Centre NPC Annual Financial Statements for the year ended 31 March 2021”, which includes the Directors’ Report as required by the Companies Act of South Africa. The other information does not include the financial statements and our auditor’s report thereon.

Our opinion on the financial statements does not cover the other information and we do not express an audit opinion or any form of assurance conclusion thereon.

In connection with our audit of the financial statements, our responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the financial statements or our knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work we have performed, we conclude that there is a material misstatement of this other information, we are required to report that fact. We have nothing to report in this regard.

Responsibilities of the directors for the financial statements

The directors are responsible for the preparation and fair presentation of the financial statements in accordance with the International Financial Reporting Standard for Small and Medium-sized Entities and the requirements of the Companies Act of South Africa, and for such internal control as the directors determine is necessary to enable the preparation of financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, the directors are responsible for assessing the Company’s ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless the directors either intend to liquidate the Company or to cease operations, or have no realistic alternative but to do so.

Auditor’s responsibilities for the audit of the financial statements

Our objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor’s report that includes our opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with ISAs will always detect a material misstatement when it exists. Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these financial statements.

As part of an audit in accordance with ISAs, we exercise professional judgement and maintain professional scepticism throughout the audit. We also:

- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for our opinion. The risk of



not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.

- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Company's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by the directors.
- Conclude on the appropriateness of the directors' use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on the Company's ability to continue as a going concern. If we conclude that a material uncertainty exists, we are required to draw attention in our auditor's report to the related disclosures in the financial statements or, if such disclosures are inadequate, to modify our opinion. Our conclusions are based on the audit evidence obtained up to the date of our auditor's report. However, future events or conditions may cause the Company to cease to continue as a going concern.
- Evaluate the overall presentation, structure and content of the financial statements, including the disclosures, and whether the financial statements represent the underlying transactions and events in a manner that achieves fair presentation.

We communicate with the directors regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that we identify during our audit.

A stylized, cursive signature of 'PricewaterhouseCoopers Inc.' in black ink.

PricewaterhouseCoopers Inc.

Director: Raj Dhanlall

Registered Auditor

Waterfall City, Johannesburg, South Africa

30 September 2021

STATEMENT OF FINANCIAL POSITION

FIGURES IN R

Figures in R	2021	2020
Assets		
Non-current assets		
Property, plant and equipment	11 031 164	3 846 044
Total non-current assets	11 031 164	3 846 044
Current assets		
Trade and other receivables	1 347 789	831 816
Cash and cash equivalents	43 889 430	29 479 356
Total current assets	45 237 219	30 311 172
Total assets	56 268 383	34 157 216
Equity and liabilities		
Equity		
Accumulated surplus	7 535 542	3 739 619
Liabilities		
Current liabilities		
Provisions	649 576	450 766
Trade and other payables	7 524 897	2 828 688
Deferred income	40 558 368	27 138 143
Total current liabilities	48 732 841	30 417 597
Total liabilities	48 732 841	30 417 597
Total equity and liabilities	56 268 383	34 157 216



STATEMENT OF SURPLUS OR DEFICIT AND OTHER COMPREHENSIVE INCOME

FIGURES IN R

	2021	2020
Revenue	72 461 711	42 311 738
Other income	5 555 478	2 940 061
Administrative expenses	(4 266 463)	(1 788 189)
Other expenses	(69 604 068)	(40 420 033)
Surplus from operating activities	4 146 658	3 043 577
Finance costs	(350 735)	(127 743)
Surplus for the year	3 795 923	2 915 834



STATEMENT OF CHANGES IN EQUITY

FIGURES IN R

	Accumulated surplus
Balance at 1 April 2019	823 785
Changes in equity	
Surplus for the year	2 915 834
Total comprehensive income	2 915 834
Balance at 31 March 2020	3 739 619
 Balance at 1 April 2020	 3 739 619
Changes in equity	
Surplus for the year	3 795 923
Total comprehensive income	3 795 923
Balance at 31 March 2021	7 535 542

ANNUAL REPORT 2020/21
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HIGHER HEALTH

Higher Education & Training: Health, Wellness and Development Centre



**higher education
& training**
Department:
Higher Education and Training
REPUBLIC OF SOUTH AFRICA



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